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Las Vegas NV 89103
Phone: (702)416-0611 Fax: (702)899-4479
NPI: 1720712235

STAFF INFO
Name: Berberova, Emiliya RN
Patient INFO
Name: ROOT, ELIZABETH
DOB: 1945-04-20

OASIS Assessment: OASIS E1 - START OF CARE

ADMINISTRATIVE INFORMATION

(M0010) Agency Medicare Provider Number
297328

(M0014) Branch State
Branch State **Nevada**

(M0016) Branch ID Number

(M0018) Attending Physician (NPI)
1265490585

Physician
LEIFRIED, STACY

Adddress
2370 CORPORATE CIR STE 300 HENDERSON NV 89074

☐ Not Applicable

(M0020) Patient ID number
123964

(M0030) Start of Care Date
03/30/2026

(M0032) Resumption of Care Date
mm/dd/yyyy

☑ Not Applicable

(M0040) Patient Name

First Name	Middle Name	Last Name	Suffix
ELIZABETH		ROOT	

(M0050) Patient State of Residence
State of Residence **Nevada**

(M0060) Patient Zip Code
89148

(M0063) Medicare Number
5WF9VD5JE16

☐ Not Applicable

(M0064) Social Security Number

☑ Not Applicable

(M0065) Medicaid Number

☑ Not Applicable

(M0066) Birth Date *
04/20/1945

(M0069) Gender *
Gender **Female**

(A1005) Ethnicity Are you of Hispanic, Latino/a, or Spanish origin? Check all that apply

- ☑ A. No, not of Hispanic, Latino/a, or Spanish origin
- ☐ B. Yes, Mexican, Mexican American, Chicano/a
- ☐ C. Yes, Puerto Rican
- ☐ D. Yes, Cuban
- ☐ E. Yes, Another Hispanic, Latino, or Spanish origin
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond

(A1010) Race What is your race?

- ☑ A. White
- ☐ B. Black or African American
- ☐ C. American Indian or Alaska Native
- ☐ D. Asian Indian
- ☐ E. Chinese
- ☐ F. Filipino
- ☐ G. Japanese
- ☐ H. Korean
- ☐ I. Vietnamese
- ☐ J. Other Asian
- ☐ K. Native Hawaiian
- ☐ L. Guamanian or Chamorro
- ☐ M. Samoan
- ☐ N. Other Pacific Islander
- ☐ X. Patient unable to respond
- ☐ Y. Patient declines to respond
- ☐ None of the above

(M0150) Current Payment Sources for Home Care: (Mark all that apply.) *

- ☐ None; no charge for current services
- ☑ Medicare (Traditional fee-for-service)
- ☐ Medicare (HMO/managed care/Advantage plan)
- ☐ Medicaid (Traditional fee-for-service)
- ☐ Medicaid (HMO/managed care)
- ☐ Worker's compensation
- ☐ Title programs (for example, Title III, V, or XX)
- ☐ Other government (for example, TriCare, VA)
- ☐ Private insurance
- ☐ Private HMO\/managed care
- ☐ Self-pay
- ☐ Other (specify)
- ☐ Unknown

A1110. Language

A.What is your preferred language?
English

B. Do you need or want an interpreter to communicate with a doctor or health care staff?
● 0 - No ○ 1 - Yes ○ 9 - Unable to determine

(M0080) Discipline Completing Assessment
● RN ○ PT ○ SLP/ST ○ OT

Assigned Staff
Berberova, Emiliya RN

(M0090) Date Assessment Completed:
03/30/2026

(M0100) Assessment Reason

- [x] Start of Care

(M0102) Date of Physician-ordered Start of Care (Resumption of Care)

If the physician indicated a specific start of care (resumption of care) date when the patient was referred for home health services, record the date specified.
Skip to A1250, Transportation, if date entered

03/30/2026

(M0104) Date of Referral:

Indicate the date that the written or verbal referral for initiation or resumption of care was received by the HHA

mm/dd/yyyy

- [ ] NA - No specific SOC/ROC date ordered by physician

(A1250) Transportation Has lack of transportation kept you from medical appointments, meetings, work, or from getting things needed for daily living?

- [x] A - Yes, it has kept me from medical appointments or from getting my medications
- [x] B - Yes, it has kept me from non-medical meetings, appointments, work, or from getting things that I need
- [ ] C - No
- [ ] X - Patient unable to respond
- [ ] Y - Patient declines to respond

(M1000) Inpatient Facilities that patient was discharged within the past 14 days? (Mark all that apply.)

- [ ] 1 - Long-term nursing facility (NF)
- [ ] 2 - Skilled nursing facility (SNF/TCU)
- [ ] 3 - Short-stay acute hospital (IPPS)
- [ ] 4 - Long-term care hospital (LTCH)
- [ ] 5 - Inpatient rehabilitation hospital or unit (IRF)
- [ ] 6 - Psychiatric hospital or unit
- [ ] 7 - Other (specify)
- [x] NA - Patient was not discharged from an inpatient facility _[Go to M1021]

(M1005) Inpatient Discharge Date (most recent)

mm/dd/yyyy

- [ ] UK - Unknown

Primary Reason for Home Health / Medical Necessity

The patient is considered homebound because she is an 80-year-old female with severe functional and medical impairments that prevent safe, independent leaving of the home without assistance. Objective findings from the record document morbid obesity (BMI 52.4), bilateral primary osteoarthritis of the knees with marked functional decline, bilateral lower-extremity lymphedema/venous stasis with dependent edema, painful bilateral foot calluses, chronic hypoxic respiratory failure/COPD (on supplemental oxygen as needed), and chronic kidney disease stage 3. Functionally she uses a walker and reports she is only able to ambulate a few steps, is unable to stand for more than a few seconds, becomes short of breath and experiences exertional lower-extremity pain with ambulation, and requires a power chair for intra-home mobility (current power chair is too large for safe use at home and she is in the process of obtaining an appropriately sized chair). She is able to transfer but has very limited ability to ambulate and is at high fall risk due to extreme obesity, knee osteoarthritis, lymphedema, neuropathic pain, and deconditioning. The patient lives alone with no live-in caregiver (friend provides near-daily assistance), which increases safety risk if she were to leave home and require unassisted ambulation or recovery from exertion. Leaving the home requires a considerable and taxing effort because even minimal ambulation provokes dyspnea and lower-extremity pain, she can only take a few steps and cannot sustain standing long enough to perform transfers or traverse typical community distances, and her current mobility equipment is inadequate for safe community ambulation. Exiting the home would therefore require either multiple persons and/or specialized equipment (appropriately sized power chair and transport) and poses significant fall and respiratory risk. Allowable, limited absences are restricted to medically necessary appointments or brief tolerated activities with appropriate transport and assistance; prolonged or frequent community outings would exceed her functional tolerance and increase risk of decompensation. Given these documented impairments (morbid obesity, bilateral knee osteoarthritis, bilateral lower-extremity lymphedema with dependent edema, chronic hypoxic respiratory failure/COPD, CKD stage 3, painful foot calluses, inability to ambulate more than a few steps, requirement for power mobility, and lack of a live-in caregiver), skilled home health services are medically necessary to provide skilled nursing for medication adjustment and renal monitoring after diuretic change, supervised management of oxygen therapy and respiratory symptoms, and skilled PT/OT for strengthening, gait/transfer training, power-chair assessment and training, and fall-risk mitigation, as well as skilled wound/podiatry coordination for foot calluses—services required to restore or maintain function and to safely manage her chronic medical conditions at home.

Homebound Reason

- [ ] Document as Clinical Narrative
- [ ] Patient is not Homebound

Criteria One (One must be met): *Describe what assistive devices the patient is dependent on (if any) and specifically why the patient condition warrants the special transportation or the assistance of another person or describe the condition that makes leaving home medically contraindicated.*

Criteria 1 A: Because of illness or injury

- [x] Patient needs the aid of supportive devices
  - [ ] Crutches
  - [ ] Cane
  - [ ] Manual Wheel Chair
  - [ ] Walker
  - [x] Motorized Wheelchair
  - [ ] Scooter
- [x] Patient Needs the use of special transportation.
- [x] Patient needs the assistance of another person to leave the place of residence

Criteria 1 B: Has a condition such as leaving his or her home is medically contraindicated.

- [ ] Status post surgery
- [ ] Angina upon minimal exertion
- [ ] Immuno deficiency

Other Homebound Reason :

Criteria Two (Both must be met)

Criteria 2 A: There must be exist a normal inability to leave home.

- [ ] Dyspnea on minimal exertion
- [ ] Bedbound
- [x] Chair bound
- [ ] Dementia
- [ ] Blindness
- [ ] Angina upon minimal exertion
- [ ] Paralysis
  - [ ] Quadriplegic or
  - [ ] Paraplegic or
  - [ ] Hemiplegia
- [x] Unable to tolerate standing or walking for extended periods
- [x] Experiencing pain impairs/restricts mobility
- [ ] Balance impaired with potential for falls; uses walls and furniture to ambulate
- [ ] Weakness and pain from recent surgery
- [ ] Visually impaired-requires constant attendance/assist
- [ ] Cognitively impaired, unable to safely leave home without assist

- [x] Leaving home is exhausting, extended rest periods to recover
- [x] Extended respiratory recovery after exertion
- [x] Requires human assist due to unsteady gait/poor balance
- [ ] Infectious disease process causing weakness and exhaustion
- [ ] Narcotics impaired ability to safely leave home unassisted
- [ ] Bedbound and requires EMS support

Other Homebound Reason :

Criteria 2 B: Leaving home must require a considerable and taxing effort

- [x] Special transportation
- [ ] Severe pain on ambulation
- [ ] Leaving familiar environment causes fear and confusion
- [x] Leaving home is strenous requiring extended rest to recover
- [ ] Fatigue to arrange/use appropriate transportation
- [x] Requires assistive device to ambulate
- [ ] Exhausting to navigate vehicle and require human assist
- [ ] Requires assist to manage equipment and public transportation
- [ ] Needs assistance to handle and transport equipment
- [x] Activities restricted due to pain limitations

Other Homebound Reason :

Prognosis

○ 1-Poor ○ 2-Guarded ● 3-Fair ○ 4-Good ○ 5-Excellent

Functional Limitations

○ 1 - Amputation
● 2 - Bowel/Bladder (Incontinence)
○ 3 - Contracture
● B- Other (Specify)
○ 4 - Hearing
○ 5 - Paralysis
● 6 - Endurance
● 7 - Ambulation
○ 8 - Speech
○ 9 - Legally blind
○ A - Dyspnea with minimal exertion

Dyspnea with moderate exertion, Severe bilateral lower extremity edema

Emergency Preparedness

Educational materials provided to suggest/assist with emergency management/decision making priorities
List of local and state approved evacuation routes and community shelters relevant to the patient's specific geographic location
Obtained the patient's emergency contact numbers for the medical record
Written materials to restate/reinforce the emergency preparedness procedures given to the Patient, and Representative.
Notified local utility companies and local emergency offices notified of life supporting equipment being used
Discussed the development of the patient's individualized emergency preparedness plan of care, including self-care readiness and the procedure to follow up with the HHA in the event services are interrupted
Discussed the HHA's plans for supporting their patients during a natural or man-made disaster
State and local emergency preparedness officials notified about the possible need for evacuation
Discussed patient specific emergency planning options
List of recommended items to have prepared/ready and available in the event of an emergency

Acuity Level

Medium Risk or Medium Priority

Emergency Triage

Category 3 - Visit could be postponed 24-48 hours without adverse effects

Additional Emergency Preparedness Information

- [x] Needs assistance during an emergency
- [x] Contact made with local/state emergency preparedness officials regarding patient in need of help during an evacuation
- [x] Medical Needs/Equipment (i.e. bedbound, oxygen, vent, IV cardiac meds other DME)

Comments

Advance Directives

Patient has Advance Directives? ○ Yes ● No

If you have checked "yes", fill the following fields:

Advance Directives (select the entities that apply to you): ○ Do Not Resuscitate (DNR) ○ Living Will

☐ Medical Power of Attorney Name Phone

Other

Copies on file at agency? ○ Yes ● No

Has surrogate? ○ Yes ● No

Name Phone

Patient was provided verbal and written information on Advance Directives? ● Yes ○ No

Is the Patient DNR (Do not Resuscitate)? ○ Yes ● No

Cultural

Primary Language? ☑ English ☐ Spanish ☐ Chinese ☐ Vietnamese Other

Is religion important to the patient? ○ Yes ○ No

Patient's religious preference:

Patient's primary source of emotional support:

Friend

VITAL SIGNS

(M1060) Height and Weight - While measuring, if the number is X.1-X.4 round down; X.5 or greater round up

a. Height (in inches). Record most recent height measure since the most recent SOC/ROC

63

b. Weight (in pounds). Base weight on most recent measure in last 30 days; measure weight consistently, according to standard agency practice (for example, in a.m. after voiding, before meal, with shoes off, etc.

270

BMI
47.82

Comments

Vital Signs

Temperature: 96.9 Temporal
Respiratory (per minute): 16
Blood Pressure mm/Hg (R): /
Blood Sugar (mg/dl): Non-Fasting
O2 Sat: 95

Pulse (per minute): Apical = Radial = 64
Weight: 270 actual
Blood Pressure mm/Hg (L): 149 / 78

O2 Sat in: Room Air

NEUROLOGICAL (HEAD)

(B1300) Health Literacy

How often do you need to have someone help you when you read instructions, pamphlets, or other written material from your doctor or pharmacy?

- [x] **0. Never**
- [ ] **1. Rarely**
- [ ] **2. Sometimes**
- [ ] **3. Often**
- [ ] **4. Always**
- [ ] **7. Patient declines to respond**
- [ ] **8. Patient unable to respond**

(C0100) Should Brief Interview for Mental Status (C0200-C0500) be Conducted?

Attempt to conduct interview with all patients. ◯ **No (patient is rarely/never understood)** ● **Yes** ◯ **Not assessed/no information**

Brief Interview for Mental Status (BIMS)

(C0200) Repetition of Three Words

Ask patient: **"I am going to say three words for you to remember. Please repeat the words after I have said all three. The words are: SOCK , BLUE , and BED . Now tell me the three words."**

Number of words repeated after first attempt ◯ **None** ◯ **One** ◯ **Two** ● **Three** ◯ **Not Assessed**

After the patient's first attempt, repeat the words using cues (**"sock, something to wear; blue, a color; bed, a piece of furniture"**). You may repeat the words up to two more times.

(C0300) Temporal Orientation (Orientation to year, month, and day)

Ask patient: **"Please tell me what year it is right now?"**
A) Able to report correct year
- ◯ **0. Missed by > 5 years or no answer**
- ◯ **1. Missed by 2-5 years**
- ◯ **2. Missed by 1 year**
- ● **3. Correct**
- ◯ **Not assessed/no information**

Ask patient: **"What month are we in right now?"**
B) Able to report correct month
- ◯ **0. Missed by > 1 month or no answer**
- ◯ **1. Missed by 6 days to 1 month**
- ● **2. Accurate within 5 days**
- ◯ **Not assessed/no information**

Ask patient: **"What day of the week is today?"**
C) Able to report correct day of the week
- ◯ **0. Incorrect or no answer**
- ● **1. Correct**
- ◯ **Not assessed/no information**

(C0400). Recall

Ask patient: **"Let's go back to an earlier question. What were those three words that I asked you to repeat?" If unable to remember a word, give cue (something to wear; a color; a piece of furniture) for that word."**

A) Able to recall "sock"
- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("something to wear")**
- ● **2. Yes, no cue required**
- ◯ **Not assessed/no information**

B) Able to recall "blue"
- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("a color")**
- ● **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C) Able to recall "bed"
- ◯ **0. No – could not recall**
- ◯ **1. Yes, after cueing ("a piece of furniture")**
- ● **2. Yes, no cue required**
- ◯ **Not assessed/no information**

C0500. BIMS Summary Score

Add scores for questions C0200-C0400 and fill in total score (00-15) **Enter 99 if the patient was unable to complete the interview** **15**

(C1310) Signs and Symptoms of Delirium (from CAM©)

A. Acute Onset of Mental Status Change

Is there evidence of an acute change in mental status from the patient's baseline?
- ● **0. No**
- ◯ **1. Yes**

B. Inattention

Did the patient have difficulty focusing attention, for example, being easily distractible or having difficulty keeping track of what was being said?
- ● **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

C. Disorganized thinking

Was the patient's thinking disorganized or incoherent (rambling or irrelevant conversation, unclear or illogical flow of ideas, or unpredictable switching from subject to subject)?
- ● **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

D. Altered level of consciousness

Did the patient have altered level of consciousness, as indicated by any of the following criteria?
- vigilant – startled easily to any sound or touch □ lethargic – repeatedly dozed off when being asked questions, but responded to voice or touch
- stuporous – very difficult to arouse and keep aroused for the interview
- comatose – could not be arouse

- ● **(0) Behavior not present**
- ◯ **(1) Behavior continuously present, does not fluctuate**
- ◯ **(2) Behavior present, fluctuates (comes and goes, changes in severity)**
- ◯ **Not assessed/no information**

(M1700) Cognitive Functioning

Patient's current (day of assessment) level of alertness, orientation, comprehension, concentration, and immediate memory for simple commands
- ● **Alert/oriented, able to focus and shift attention, comprehends and recalls task directions independently.**
- ◯ **Requires prompting (cuing, repetition, reminders) only under stressful or unfamiliar conditions.**
- ◯ **Requires assistance and some direction in specific situations (for example, on all tasks involving shifting of attention) or consistently requires low stimulus environment due to distractibility**
- ◯ **Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.**
- ◯ **Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state, or delirium.**

(M1710) When Confused (Reported or Observed Within the Last 14 Days):
- ● **Never**
- ◯ **In new or complex situations only**
- ◯ **On awakening or at night only**
- ◯ **During the day and evening, but not constantly**
- ◯ **Constantly**
- ◯ **Patient nonresponsive**

(M1720) When Anxious (Reported or Observed Within the Last 14 Days):
- ● **None of the time**
- ◯ **Less often than daily**
- ◯ **Daily, but not constantly**
- ◯ **All of the time**
- ◯ **Patient nonresponsive**

Neuro/Emotional/Behavioral Assessment

Conditions of Participation
Psychosocial

- [x] **WNL (Within Normal Limits)**
- [ ] **Poor Home Environment**
- [ ] **Agitated**

- [ ] Impaired Decision Making
- [ ] Poor Coping Skills
- [ ] Demonstrated/Expressed Anxiety
- [ ] Document as Clinical Narrative
- [ ] Inappropriate Behavior
- [ ] Depressed Mood
- [ ] Irritability

Educational level

Primary Language

English

Unable to: ☐ *Read* ☐ *Write*

Barriers:

Language

- ◯ **Needs interpreter**

Learning

- ◯ **Mental**
- ◯ **Physical**
- ◯ **Psychosocial**
- ◯ **Functional**

Emotions Reported:

- ◯ **Angry**
- ◯ **Discouraged**
- ◯ **Withdrawn**
- ◯ **Flat affect**
- ◯ **Difficulty coping**
- ◯ **Disorganized**

Treatment:

- ◯ **Depressed**
- ◯ **Recent**
- ◯ **Long Term**

Evidence of abuse/neglect/exploitation:

- ◯ **Potential**
- ◯ **Verbal/Emotional**
- ◯ **Physical**
- ◯ **Actual**
- ◯ **Financial**
- ● **No Evidence of Abuse/Neglect/Exploitation**

Unable to cope with altered health status, patient:

- ◯ **Lacks motivation**
- ◯ **Has unrealistic expectations**
- ◯ **Denies problems**
- ● **Unable to recognize problems**

Sleep/Rest

- ● **Adequate**
- ◯ **Inadequate**

No. of hours slept per night: **7**

Comments

Spiritual / Cultural implications that impact care

Spiritual resource

☐ Inappropriate responses to caregivers/clinician

☐ Inappropriate follow-through in past

Did patient drive a vehicle before admission? ☐Yes ☑No ☐NA

Did the patient have a job before admission? ◯Yes ●No

If yes, do they want to drive post-discharge? ☐Yes ☐No

If yes, do they want to return to work post-discharge? ◯Yes ◯No

Mental Status:

- ● **1 - Oriented**
- ◯ **2 - Comatose**
- ◯ **3 - Forgetful**
- ◯ **4 - Depressed**
- ◯ **5 - Disoriented**
- ◯ **6 - Lethargic**
- ◯ **7 - Agitated**
- ● **8 - Alert**
- ◯ **9 - Confused**
- ◯ **10 - Dementia**
- ◯ **11 - Assaultive**
- ◯ **12 - Delirious**
- ◯ **13 - Wanders**
- 14 - Other

Describe the patient's mental status. Include general appearance, behaviors, emotional responses, mental functioning, overall social interaction as well as both clinical objective observations and subjective descriptions reported during this visit.

Patient is alert and oriented x4 and able to speak clearly and effectively. The patient verbalized feelings appropriately, demonstrating coherent and logical thought processes. Observations during the visit indicate no apparent issues with social interaction or communication. The patient appeared well-groomed, exhibited appropriate behaviors, and displayed a calm emotional demeanor. Both clinical observations and subjective reports suggest the patient has no difficulties in engaging socially or maintaining meaningful interactions with

Has there been a sudden/acute change in their mental status? ☐ Yes ☐ No Reported by: ☐ If yes, did the change coincide with something else like medication change, fall, or loss of a loved one? ☐ No ☐ Yes

Comments:

Neuromuscular

Impairment:

- ☑ **No Problems identified**
- ☐ **Speech**
- ☐ **Hearing**
- ☐ **Visual**
- ☐ **Tremors**
- ☐ **Numbness**
- ☐ **Tingling**
- ☐ **Vertigo**
- ☐ **Ataxia**
- ☐ **Decreased Sensitivity**

Fall(s) since last visit (Specify):

Grip Strength:

- ☑ **Normal**
- ☐ **Strong**
- ☐ **Weak**
- ☑ **Equal**
- ☐ **Unequal**
- ☐ **Bilateral**
- ☐ **Right**
- ☐ **Left**

Others (Specify):

Grasp Strength:

- ☑ **Normal**
- ☐ **Strong**
- ☐ **Weak**
- ☑ **Equal**
- ☐ **Unequal**

Mobility:

- [ ] Balance Within Normal Limits
- [x] Unsteady Gait

Weakness (Specify):
Generalized weakness

Change in ADL (Explain):
Patient has the following limitations on ambulation and mobility due to weakness, SOB, and pain that require the assistance of another person for the patient to safely perform ADLs.

EYES AND EARS

B0200. Hearing

Ability to hear (with hearing aid or hearing appliances if normally used)

- [x] **0. Adequate – no difficulty in normal conversation, social interaction, listening to TV**
- [ ] **1. Minimal difficulty – difficulty in some environments (e.g., when person speaks softly, or setting is noisy)**
- [ ] **2. Moderate difficulty – speaker has to increase volume and speak distinctly**
- [ ] **3. Highly impaired – absence of useful hearing**

B1000. Vision

Ability to see in adequate light (with glasses or other visual appliances)

- [x] **0. Adequate – sees fine detail, such as regular print in newspapers/books**
- [ ] **1.Impaired – sees large print, but not regular print in newspapers/books**
- [ ] **2.Moderate impaired – limited vision; not able to see newspaper headlines but can identify objects**
- [ ] **3.Highly impaired – object identification in question, but eyes appear to follow objects**
- [ ] **4.Severely impaired – no vision or sees only light, colors or shapes; eyes do not appear to follow objects**

Pupils

- [ ] No Problems Identified
- [x] PERRLA
- [ ] Anisocoria
- [ ] Right
- [ ] Left
- [ ] Others(Specify):

Vision

- [ ] No Problems Identified
- [x] Glasses
- [ ] Macular Degeneration
- [ ] Blurred Vision
- [ ] Glaucoma
- [ ] Contact Lenses:
- [ ] Right
- [ ] Left
- [ ] Cataracts
- [ ] Blindness(Specify):
- [ ] Others (Specify):

Hearing

- [x] No Problems Identified
- [ ] Right Hearing Impairment
- [ ] Left Hearing Impairment
- [ ] Deaf
- [ ] Speech Impairment (Specify):

CARDIOVASCULAR

Heart Sounds

- [ ] Normal
- [ ] Regular
- [x] Irregular
- [ ] Murmur
- [ ] Gallops
- [ ] Clicks
- [ ] Friction Rule

Comments (If Abnormal (Explain)):

Edema

Upper:

- [ ] Right Extremities
- [ ] Left Extremities

Lower:

- [x] Right Extremities
- [x] Left Extremities

Others:

Comments:
Patient with severe lymphedema on both lower extremities; no open areas were noted. A podiatry referral was made. The patient was instructed on measures to relieve edema, such as elevating the legs when sitting to improve blood circulation.

.

- [ ] Cramps
- [ ] Claudication
- [x] Fatigue

Capillary Refill:

- [ ] > 3 secs
- [x] < 3 secs

PT/INR: /

Peripheral Pulses

Radial:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent
Pedal:	[x] Strong/Bounding	[ ] Weak	[ ] Thready	[ ] Absent

Chest Pain

- [x] Denies
- [ ] Anginal
- [ ] Postural
- [ ] Localized
- [ ] Substernal
- [ ] Sharp
- [ ] Dull
- [ ] Aching
- [ ] Radiating
- [ ] Viselike
- [ ] Stabbing
- [ ] Others (Specify):

Associated with:

- [x] Shortness of Breath
- [x] Activity
- [ ] Rest
- [ ] Frequency/Duration:

Comments:
SN educated patient on s/s of cardiopulmonary dysfunction and actions to take to mitigate symptoms and prevent worsening. Seek immediate medical attention for severe or sudden symptoms: Call 911 or your local emergency number immediately for sudden, severe chest pain, shortness of breath, fainting, dizziness, or changes in vision or speech.

RESPIRATORY (CHEST/THORAX)

M1400. When is the patient dyspneic or noticeably Short of Breath?

- [ ] **0. Patient is not short of breath**
- [ ] **1. When walking more than 20 feet, climbing stairs**
- [x] **2. With moderate exertion (for example, while dressing, using commode or bedpan, walking distances less than 20 feet)**
- [ ] **3. With minimal exertion (for example, while eating, talking, or performing other ADLs) or with agitation**
- [ ] **4. At rest (during day or night)**

Respiratory Status

- [x] Dyspnea:
- [ ] At Rest
- [x] With Exertion/Activity
- [ ] Accessory Muscle Used
- [ ] Orthopnea
- [ ] Retraction
- [ ] Stridor

O2 3 lpm O2 Saturation 95 % ☐ PRN ☑ Continuous

☐ Mechanical Ventilator: ☐ Intubated ☐ Tracheostomy Others (Specify):

Lung Sounds

Anterior: Right: **Clear** Left: **Clear**
Posterior: Upper Right: **Clear** Upper Left: **Clear** Lower Right: **Clear** Lower Left: **Clear**

Cough

☑ None ☐ Dry ☐ Acute ☐ Chronic ☐ Non-Productive

☐ Productive: ☐ Not Assessed/No Information ☐ Yes ☐ No Suction: ☐ Not Assessed/No Information ☐ Yes ☐ No

☐ Hemoptysis Frequency: Amount:

GASTROINTESTINAL (MOUTH TO ANUS)

K0520. Nutritional Approaches

	On Admission (Check All That Apply)
A. Parenteral/IV feeding	☐
B. Feeding tube (e.g., nasogastric or abdominal (PEG))	☐
C. Mechanically altered diet – require change in texture of food or liquids (e.g., pureed food, thickened liquids)	☐
D. Therapeutic diet (e.g., low salt, diabetic, low cholesterol)	☑
Z. None of the above	☐

(M1870) Feeding or Eating

Current ability to feed self meals and snacks safely. Note: This refers only to the process of ***eating***, ***chewing***, and ***swallowing***, not preparing the food to be eaten.

- ○ Able to independently feed self.
- ● Able to feed self independently but requires: (a) meal set-up; OR (b) intermittent assistance or supervision from another person; OR (c) a liquid, pureed or ground meat diet.
- ○ Unable to feed self and must be assisted or supervised throughout the meal/snack.
- ○ Able to take in nutrients orally and receives supplemental nutrients though a nasogastric tube or gastrostomy.
- ○ Unable to take in nutrients orally and is fed nutrients through a nasogastric tube or gastrostomy.
- ○ Unable to take in nutrients orally or by tube feeding.

(M1620) Bowel Incontinence Frequency

- ● Very rarely or never has bowel incontinence
- ○ Less than once weekly
- ○ One to three times weekly
- ○ Four to six times weekly
- ○ On a daily basis
- ○ More often than once daily
- ○ Patient has ostomy for bowel elimination
- ○ Unknown

(M1630) Ostomy for Bowel Elimination

Does this patient have an ostomy for bowel elimination that (within the last 14 days): a) was related to an inpatient facility stay; or b) necessitated a change in medical or treatment regimen?

- ● Patient does not have an ostomy for bowel elimination
- ○ The ostomy was related to an inpatient stay or did necessitate change in medical or treatment regimen.
- ○ Patient's ostomy was not related to an inpatient stay and did not necessitate change in medical or treatment regimen.

Mouth and Throat

☑ No Problems Identified

☐ Dentures: ☐ Upper ☐ Lower ☐ Partial

☐ Bleeding Gums/Gingivitis ☐ Toothache ☐ Ulcerations ☐ Lesion

☐ Dysphagia ☐ Hoarseness ☐ Sore Throat ☐ Masses/Tumors

Comments:

Appetite

☑ Good ☐ Fair ☐ Poor Intake: ☑ Adequate ☐ Inadequate

Abnormalities:

☐ Anoxeria ☐ Cachexia ☐ Nausea ☐ Vomiting

☐ Decreased Appetite ☐ Reflux/Indigestion ☐ Diarrhea ☐ Constipation

☐ Bowel Incontinence ☐ Other (Specify):

Feeding

☐ Formula ☐ Continuous Feeding cc hours

☐ Bolus: ☐ Gravity ☐ Pump cc every hour(s)

Flushing Protocol:

PEG/G-Tube Placement:

☐ Placement Checked ☐ Residual Checked Amount:

#16 Nutritional Requirements

☐ NPO ☑ No Added Salt ☐ Low Salt ☑ Low Fat ☑ Low Cholesterol ☐ No Concentrated Sweets

☑ Controlled Carbohydrates ☐ Regular ☑ Fluid Restriction:

Other:

Nutritional Health Screen	Score	Nutritional Health Screen	Score
○ Without reason, has lost more than 10 lbs in the last 3 months	15	○ Does not always have enough money to buy foods need	10
		Total	20

Nutritional Health Screen	Score	Nutritional Health Screen	Score
● Has an illness or condition that made pt change the type and/or amount of food eaten	10	● Eats alone most of the time	5
○ Has open decubitus, ulcer, burn or wound	10		
○ Eats fewer than 2 meals a day	10	● Takes 3 or more prescribed or OTC medications a day	5
○ Has a tooth/mouth problem that makes it hard to eat	10	○ Is not always physically able to cook and/or feed self and has no caregiver to assist	5
○ Has 3 or more drinks of beer, liquor or wine almost every day	10	○ Frequently has a diarrhea or constipation	5
		Total	20

● Good Nutritional Status (Score 0 - 25) ○ Moderate Nutritional Status (Score 25 - 54) ○ High Nutritional Status (Score 55 - 100)

Nutritional Status Commments:
SN educated the patient on the prescribed diet and the importance of proper nutrition and hydration.

Weight

☐ No Change ☐ Weight Gain ☑ Weight Loss

Abdomen

☑ Abdominal Soft/Tender ☐ Abdominal Distention ☐ Girth(cm)

Ostomy/Stoma Appearance

☐ Pinkished-Red ☐ Moist Abnormalities: ☐ Purple ☐ Black in Color ☐ Dry
☐ Gray ☐ Bad Odor ☐ Itches/Hurt Other (Specify):

Bowel Sounds:

Abdominal Quadrants:

☑ Right Lower	☑ Right Upper	☑ Left Lower	☑ Left Upper	Last Bowel Movement:
Normoactive	Normoactive	Normoactive	Normoactive	03/29/2026

Constipation

☐ Chronic ☐ Acute ☐ Occasional ☐ Internal Hemorrhoids
☐ External Hemorrhoids ☐ Laxative/Enema Use

Stool Appearance

☑ Within Normal Limits ☐ Gray ☐ Tarry ☐ Black ☐ Fresh Blood
☐ Others (Specify):

GENITOURINARY (GENITALS AND URINARY)

(M1600) Has this patient been treated for a Urinary Tract Infection in the past 14 days?

● No ○ Patient on prophylactic treatment
○ Yes ○ Unknown

(M1610) Urinary Incontinence or Urinary Catheter Presence:

● Patient is incontinent ○ No incontinence or catheter (includes anuria or ostomy for urinary drainage)
- ○ Day time
- ○ Night time
- ○ Day and Night
- ○ Timed-voiding defers incontinence
- ● Occasional stress incontinence

○ Patient requires a urinary catheter (specifically: external, indwelling, intermittent, or suprapubic)

Genitourinary Assessment

☐ Within Normal Limits

Abnormalities:

☑ Frequency ☐ Dysuria ☑ Urgency ☐ Anuria ☐ Nocturia ☐ Oliguria
☐ Retention ☐ Hematuria ☐ Irrigation ☐ Incontinence ☐ Diapers Used ☐ Penile Discharge
☐ Vaginal Bleeding/Discharge Others (Specify):

Urine Color:

☑ Clear ☐ Transparent ☐ Pale Yellow ☐ Brownish Orange
☐ Pinkish Red ☐ Dark Yellow ☐ Blue or Green ☐ Foamy
☑ Transparent Yellow

Urine Consistency:

☐ Hematuria ☐ Sediment ☐ Odorous ☐ Cloudy

Urinary Catheter:

☐ External Catheter ☐ Foley ☐ Suprapubic ☐ Nephrostomy

Last Catheter Change: mm/dd/yyyy Fr: cc:

MUSCULOSKELETAL

GG0100. Prior Functioning: Everyday Activities: Indicate the patient's usual ability with everyday activities prior to the current illness, exacerbation, or injury.
CODING:

- **3. Independent** – Patient completed the activities by him/herself, with or without an assistive device, with no assistance from a helper
- **2. Needed Some Help** – Patient needed partial assistance from another person to complete activities
- **1. Dependent** – A helper completed the activities for the patient.
- **8. Unknown**
- **9. Not Applicable**

	Independent	Needed Some Help	Dependent	Unknown	Not Applicable
A. Self Care: Code the patient's need for assistance with bathing, dressing, using the toilet, or eating prior to the current illness, exacerbation, or injur	✓				
B. Indoor Mobility (Ambulation): Code the patient's need for assistance with walking from room to room (with or without a device such as cane, crutch or walker) prior to the current illness, exacerbation, or injury.		✓			
C. Stairs: Code the patient's need for assistance with internal or external stairs (with or without a device such as cane, crutch, or walker) prior to the current illness, exacerbation or injury.		✓			
D. Functional Cognition: Code the patient's need for assistance with planning regular tasks, such as shopping or remembering to take medication prior to the current illness, exacerbation, or injury.	✓				

(GG0110) Prior Device Use

Indicate devices and aids used by the patient prior to the current illness, exacerbation, or injury (**Check all that apply)**

- [ ] **A. Manual wheelchair**
- [ ] **B. Motorized wheelchair and/or scooter**
- [ ] **C. Mechanical lift**
- [x] **D. Walker**
- [ ] **E. Orthotics/Prosthetics**
- [ ] **Z. None of the above**

GG0170 - MOBILITY

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Coding:
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.
If activity was not attempted, code reason:
07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0170) A. Roll left and right: The ability to roll from lying on back to left and right side, and return to lying on back on the bed.
SOC/ROC Performance

- ( ) **(06) Independent**
- ( ) **(05) Setup or clean-up assistance**
- (●) **(04) Supervision or touching assistance**
- ( ) **(03) Partial/moderate assistance**
- ( ) **(02) Substantial/maximal assistance**
- ( ) **(01) Dependent**
- ( ) **(07) Patient refused**
- ( ) **(09) Not applicable**
- ( ) **(10) Not attempted due to environmental limitations**
- ( ) **(88) Not attempted due to medical conditions or safety concerns**
- ( ) **(-) Not assessed/no information**

(GG0170) B. Sit to lying: The ability to move from sitting on side of bed to lying flat on the bed.
SOC/ROC Performance

- ( ) **(06) Independent**
- ( ) **(05) Setup or clean-up assistance**
- (●) **(04) Supervision or touching assistance**
- ( ) **(03) Partial/moderate assistance**
- ( ) **(02) Substantial/maximal assistance**
- ( ) **(01) Dependent**
- ( ) **(07) Patient refused**
- ( ) **(09) Not applicable**
- ( ) **(10) Not attempted due to environmental limitations**
- ( ) **(88) Not attempted due to medical conditions or safety concerns**
- ( ) **(-) Not assessed/no information**

(GG0170) C. Lying to sitting on side of bed: The ability to move from lying on the back to sitting on the side of the bed with feet flat on the floor, and with no back support.
SOC/ROC Performance

- ( ) **(06) Independent**
- ( ) **(05) Setup or clean-up assistance**
- ( ) **(04) Supervision or touching assistance**
- (●) **(03) Partial/moderate assistance**
- ( ) **(02) Substantial/maximal assistance**
- ( ) **(01) Dependent**
- ( ) **(07) Patient refused**
- ( ) **(09) Not applicable**
- ( ) **(10) Not attempted due to environmental limitations**
- ( ) **(88) Not attempted due to medical conditions or safety concerns**
- ( ) **(-) Not assessed/no information**

(GG0170) D. Sit to stand: The ability to come to a standing position from sitting in a chair, wheelchair, or on the side of the bed.
SOC/ROC Performance

- ( ) **(06) Independent**
- ( ) **(05) Setup or clean-up assistance**
- ( ) **(04) Supervision or touching assistance**
- (●) **(03) Partial/moderate assistance**
- ( ) **(02) Substantial/maximal assistance**
- ( ) **(01) Dependent**
- ( ) **(07) Patient refused**
- ( ) **(09) Not applicable**
- ( ) **(10) Not attempted due to environmental limitations**
- ( ) **(88) Not attempted due to medical conditions or safety concerns**
- ( ) **(-) Not assessed/no information**

(GG0170) E. Chair/bed-to-chair transfer: The ability to transfer to and from a bed to a chair (or wheelchair).
SOC/ROC Performance

- ( ) **(06) Independent**
- ( ) **(05) Setup or clean-up assistance**
- ( ) **(04) Supervision or touching assistance**
- (●) **(03) Partial/moderate assistance**
- ( ) **(02) Substantial/maximal assistance**
- ( ) **(01) Dependent**
- ( ) **(07) Patient refused**
- ( ) **(09) Not applicable**
- ( ) **(10) Not attempted due to environmental limitations**
- ( ) **(88) Not attempted due to medical conditions or safety concerns**
- ( ) **(-) Not assessed/no information**

(GG0170) F. Toilet transfer: The ability to get on and off a toilet or commode.
SOC/ROC Performance

- ( ) **(06) Independent**
- ( ) **(05) Setup or clean-up assistance**
- ( ) **(04) Supervision or touching assistance**
- (●) **(03) Partial/moderate assistance**
- ( ) **(02) Substantial/maximal assistance**
- ( ) **(07) Patient refused**
- ( ) **(09) Not applicable**
- ( ) **(10) Not attempted due to environmental limitations**
- ( ) **(88) Not attempted due to medical conditions or safety concerns**
- ( ) **(-) Not assessed/no information**

- ◯ (01) Dependent

(GG0170) G. Car transfer: The ability to transfer in and out of a car or van on the passenger side. Does not include the ability to open/close door or fasten seat belt.

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ● (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) I. Walk 10 feet: Once standing, the ability to walk at least 10 feet in a room, corridor, or similar space. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170M, 1 step (curb)*

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) J. Walk 50 feet with two turns: Once standing, the ability to walk 50 feet and make two turns.

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) K. Walk 150 feet: Once standing, the ability to walk at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) L. Walking 10 feet on uneven surfaces: The ability to walk 10 feet on uneven or sloping surfaces (indoor or outdoor), such as turf or gravel.

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) M. 1 step (curb): The ability to go up and down a curb and/or up and down one step. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) N. 4 steps: The ability to go up and down four steps with or without a rail. *If SOC/ROC performance is coded 07, 09, 10 or 88, skip to GG0170P, Picking up object.*

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) O. 12 steps: The ability to go up and down 12 steps with or without a rail.

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) P. Picking up object: The ability to bend/stoop from a standing position to pick up a small object, such as a spoon, from the floor.

SOC/ROC Performance

- ◯ (06) Independent
- ◯ (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ● (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) Q. Does patient use a wheelchair and/or scooter?

- ◯ (0) No **[GOTO M1600]**
- ● (1) Yes
- ◯ Not assessed (no information)

(GG0170) R. Wheel 50 feet with two turns: Once seated in wheelchair/scooter, the ability to wheel at least 50 feet and make two turns.

SOC/ROC Performance

- ◯ (06) Independent
- ● (05) Setup or clean-up assistance
- ◯ (04) Supervision or touching assistance
- ◯ (03) Partial/moderate assistance
- ◯ (02) Substantial/maximal assistance
- ◯ (01) Dependent
- ◯ (07) Patient refused
- ◯ (09) Not applicable
- ◯ (10) Not attempted due to environmental limitations
- ◯ (88) Not attempted due to medical conditions or safety concerns
- ◯ (-) Not assessed/no information

(GG0170) RR1. Indicate the type of wheelchair or scooter used.

- ( ) (1) Manual
- (✓) (2) Motorized
- ( ) Not assessed (no information)

(GG0170) S. Wheel 150 feet: Once seated in wheelchair/scooter, the ability to wheel at least 150 feet in a corridor or similar space.

SOC/ROC Performance

- ( ) (06) Independent
- (✓) (05) Setup or clean-up assistance
- ( ) (04) Supervision or touching assistance
- ( ) (03) Partial/moderate assistance
- ( ) (02) Substantial/maximal assistance
- ( ) (01) Dependent
- ( ) (07) Patient refused
- ( ) (09) Not applicable
- ( ) (10) Not attempted due to environmental limitations
- ( ) (88) Not attempted due to medical conditions or safety concerns
- ( ) (-) Not assessed/no information

(GG0170) SS1. Indicate the type of wheelchair or scooter used.

- ( ) (1) Manual
- (✓) (2) Motorized
- ( ) Not assessed (no information)

Gait and Mobility Abnormalities

- [ ] No Problems Identified
- [x] Weakness
- [ ] Bed Bound
- [ ] Joint Stiffness
- [ ] Orthopedic Aftercare
- [ ] Weight Bearing Restriction
- [ ] Fracture:
- [ ] Amputation:
- [ ] Paresthesia:
- [ ] Foot Drop: [ ] Right [ ] Left
- [x] Poor Balance
- [x] Chair Bound
- [ ] Shuffling/Wide-based Gait
- [x] High Risk for Falls
- [x] Poor Conditioning
- [x] Joints Pain: Bilateral Knees
- [ ] Atrophy:
- [ ] Contractures: Site

Range of Motion

- [ ] Limited Range of Motion: [ ] RUE [ ] RLE [ ] LUE [ ] LLE
- [ ] Quadriplegia
- [ ] Hemiplegia
- [ ] Paraplegia

Comments:
Patient currently relies on a motorized wheelchair for ambulation and transfer and requires assistance/supervision in ADL's/IADL's. A cane and walker do not meet her needs as it does not provide enough support, and the patient is unable to ambulate more than a few steps to transfer, making her a fall risk. She can stand from a seated position without assistance and get on her current power chair. She gets short of breath with ambulation and can only ambulate a few steps due to exertion as well as lower extremity pain.

INTEGUMENTARY (SKIN)

M1306. Does this patient have at least one Unhealed Pressure Ulcer/Injury at Stage 2 or Higher or designated as Unstageable?
(Excludes Stage 1 pressure injuries and all healed pressure ulcers/injuries)

- (✓) 0. No → Skip to M1322, Current Number of Stage 1 Pressure Injuries at SOC/ROC; Skip to M1324, Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable at DC
- ( ) 1. Yes

M1311. Current Number of Unhealed Pressure Ulcers/Injuries at Each Stage

A1. Stage 2: Partial thickness loss of dermis presenting as a shallow open ulcer with a red or pink wound bed, without slough. May also present as an intact or open/ruptured blister. **Number of Stage 2 pressure ulcers**	^
B1. Stage 3: Full thickness tissue loss. Subcutaneous fat may be visible but bone, tendon, or muscle is not exposed. Slough may be present but does not obscure the depth of tissue loss. May include undermining and tunneling. **Number of Stage 3 pressure ulcers**	^
C1. Stage 4: Full thickness tissue loss with exposed bone, tendon, or muscle. Slough or eschar may be present on some parts of the wound bed. Often includes undermining and tunneling. **Number of Stage 4 pressure ulcers**	^
D1. Unstageable: Non-removable dressing/device: Known but not stageable due to non┐removable dressing/device **Number of unstageable pressure ulcers/injuries due to non-removable dressing/device**	^
E1. Unstageable: Slough and/or eschar: Known but not stageable due to coverage of wound bed by slough and/or eschar **Number of unstageable pressure ulcers due to coverage of wound bed by slough and/or eschar**	^
F1. Unstageable: Deep tissue injury Number of unstageable pressure injuries presenting as deep tissue injury	^

(M1322) Current Number of Stage 1 Pressure Injuries:
Intact skin with non-blanchable redness of a localized area usually over a bony prominence. Darkly pigmented skin may not have a visible blanching; in dark skin tones only it may appear with persistent blue or purple hues.
Stage 1 Pressure Injuries **0**

(M1324) Stage of Most Problematic Unhealed Pressure Ulcer/Injury that is Stageable:
(Excludes pressure ulcer/injury that cannot be staged due to a non-removable dressing/device, coverage of wound bed by slough and/or eschar, or deep tissue injury.)
Stage of Most Problematic **Patient has no pressure ulcers or no stageable pressure ulcers**

(M1330) Does this patient have a Stasis Ulcer?

- (✓) No **[GO TO M1340]**
- ( ) Yes, patient has BOTH observable and unobservable stasis ulcers
- ( ) Yes, patient has observable stasis ulcers ONLY
- ( ) Yes, patient has unobservable stasis ulcers ONLY (known but not observable due to non┐removable dressing/device) **[GO TO M1340]**

(M1332) Current Number of Stasis Ulcer(s) that are Observable:

- ( ) One
- ( ) Two
- ( ) Three
- ( ) Four or more

(M1334) Status of Most Problematic Stasis Ulcer that is Observable

- ( ) Fully granulating
- ( ) Early/partial granulation
- ( ) Not healing

(M1340) Does this patient have a Surgical Wound?

- (✓) No **[GO TO N0415]**
- ( ) Yes, patient has at least one observable surgical wound
- ( ) Surgical wound known but not observable due to non-removable dressing/device **[GO TO N0415]**

(M1342) Status of Most Problematic Surgical Wound that is Observable

- ( ) Newly epithelialized
- ( ) Early/partial granulation
- ( ) Fully granulating
- ( ) Not healing

Skin Color

- [x] Within Normal Limits
- [ ] Pale
- [ ] Jaundice
- [ ] Cyanotic

Skin Turgor:

☑ Good ☐ Fair ☐ Poor

Skin Abnormalities:

☑ Dry ☐ Warm ☐ Cool ☐ Rash ☐ Ulcer ☐ Ostomy

☐ Incision ☐ Diaphoretic ☐ Others (Specify):

Norton Scale for Pressure Ulcer Risk Assessment

	Good (4)	Fair (3)	Poor (2)	Very Bad (1)
Physical Condition	○ Good	● Fair	○ Poor	○ Very Bad
Mental Condition	● Alert	○ Apathetic	○ Confused	○ Stuporous
Activity	○ Ambulant	○ Walks With Help	● Chairbound	○ Bedfast
Mobility	○ Full	○ Slightly Impaired	● Very Limited	○ Immobile
Incontinence	○ None	● Occassional	○ Usually Urinary	○ Urinary and Fecal
			Total Score:	14

Wound

Fetch Wounds From Previous Wound Note

PAIN ASSESSMENT

J0510. Pain Effect on Sleep

Ask patient: "Over the past 5 days, how much of the time has pain made it hard for you to sleep at night"

○ 0. Does not apply – I have not had any pain or hurting in the past 5 days
○ 1. Rarely or not at all
● 2. Occasionally
○ 3. Frequently
○ 4. Almost constantly
○ 8. Unable to answer

J0520. Pain Interference with Therapy Activities

Ask patient: "Over the past 5 days, how often have you limited your participation in rehabilitation therapy sessions due to pain?"

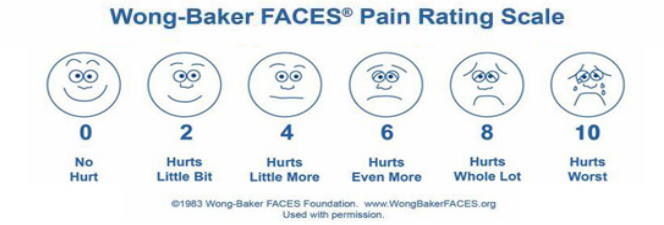
● 0. Does not apply – I have not received rehabilitation therapy in the past 5 days
○ 1. Rarely or not at all
○ 2. Occasionally
○ 3. Frequently
○ 4. Almost constantly
○ 8. Unable to answer

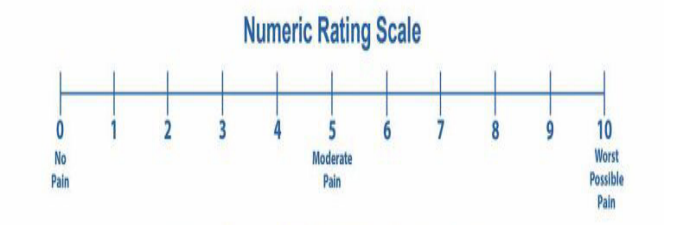
J0530. Pain Interference with Day-to-Day Activities

Ask patient: "Over the past 5 days, how often you have limited your day-to-day activities (excluding rehabilitation therapy session) because of pain?"

○ 1 - Rarely or not at all
○ 2 - Occasionally
● 3 - Frequently
○ 4 - Almost Constantly
○ Unable to answer

Pain





5

Collected Using: ☐ Face Scale ☑ Numeric Rating Scale

Pain Location and Description:

1 **Location**: Bilateral knee

Start Date: 03/30/2026

Activity: With Activity×

What makes pain worse?: Movement×

Intensity: 5×

Duration: Chronic

What makes pain better?: Med/Rest×

Does pain affect quality of life?: yes

Description: Dull Ache×

Unable to Assess ☐

Frequency: Frequent×

Unable to Assess ☐

Type of Intervention: Non Opiod Analgesic×

How well it is working: Fairly Well×

Status: COC×

EMOTIONAL/MOOD

(D0150) Patient Mood Interview (PHQ-2 to 9)

Say to patient: **"Over the last 2 weeks, have you been bothered by any of the following problems?"**
If symptom is present, **enter 1 (yes) in column 1 , Symptom Presence** . If yes in column 1, then ask the patient: **"About how often have you been bothered by this?"**
Read and show the patient a card with the symptom frequency choices. Indicate response in column 2, Symptom Frequency

1) Symptom Presence
- **0. No (enter 0 in column 2)**
- **1. Yes (enter 0-3 in column 2)**
- **9. No response (leave column 2 blank)**

2) Symptom Frequency
- 0 - **Never or 1 day**
- 1 - **2-6 days (several days)**
- 2 - **7-11 days (half or more of the days)**
- 3 - **12-14 days (nearly every day)**

	Presence	Frequency
A. Little interest or pleasure in doing things	**No** ×	**Never or 1 day**
B. Feeling down, depressed, or hopeless	**No** ×	**Never or 1 day**
If either **D0150A2 or D0150B2** is coded **2 or 3** , **CONTINUE** asking the questions below. If not, **END the PHQ interview**		
C. Trouble falling or staying asleep, or sleeping too much	^	^
D. Feeling tired or having little energy	^	^
E. Poor appetite or overeating	^	^
F. Feeling bad about yourself – or that you are a failure or have let yourself or your family down	^	^
G. Trouble concentrating on things, such as reading the newspaper or watching television	^	^
H. Moving or speaking so slowly that other people could have noticed. Or the opposite – being so fidgety or restless that you have been moving around a lot more than usual	^	^
I. Thoughts that you would be better off dead, or of hurting yourself in some way	^	^
D0160. Total Severity Score Add scores for all frequency responses in Column 2, Symptom Frequency. Total score must be between 02 and 27. Enter 99 if unable to complete interview (i.e., Symptom Frequency is blank for 3 or more required items)		**0**

D0700. Social Isolation

How often do you feel lonely or isolated from those around you?

- [x] **0 - Never**
- [ ] **1 - Rarely**
- [ ] **2 - Sometimes**
- [ ] **3 - Often**
- [ ] **4 - Always**
- [ ] **7 - Patient declines to respond**
- [ ] **8 - Patient unable to respond**

(M1740) Cognitive, behavioral, and psychiatric symptoms that are demonstrated at least once a week (Reported or Observed): (Mark all that apply.

- [ ] **1 - Memory deficit: failure to recognize familiar persons/places, inability to recall events of past 24 hours, significant memory loss so that supervision is required**
- [ ] **2 - Impaired decision-making: failure to perform usual ADLs or IADLs, inability to appropriately stop activities, jeopardizes safety through actions**
- [ ] **3 - Verbal disruption: yelling, threatening, excessive profanity, sexual references, etc.**
- [ ] **4 - Physical aggression: aggressive or combative to self and others (for example, hits self, throws objects, punches, dangerous maneuvers with wheelchair or other objects)**
- [ ] **5 - Disruptive, infantile, or socially inappropriate behavior (excludes verbal actions)**
- [ ] **6 - Delusional, hallucinatory, or paranoid behavior**
- [x] **7 - None of the above behaviors demonstrated**

(M1745) Frequency of Disruptive Behavior Symptoms (Reported or Observed):

- [x] **0 - Never**
- [ ] **1 - Less than once a month**
- [ ] **2 - Once a month**
- [ ] **3 - Several times each month**
- [ ] **4 - Several times a week**
- [ ] **5 - At least daily**

FUNCTIONAL STATUS

(M1800) Grooming

Current ability to tend safely to personal hygiene needs (specifically: washing face and hands, hair care, shaving or make up, teeth or denture care, or fingernail care)

- [ ] **0 - Able to groom self unaided, with or without the use of assistive devices or adapted methods.**
- [x] **1 - Grooming utensils must be placed within reach before able to complete grooming activities.**
- [ ] **2 - Someone must assist the patient to groom self.**
- [ ] **3 - Patient depends entirely upon someone else for grooming needs**

(M1810)

Current **Ability to Dress Upper Body** safely (with or without dressing aids) including undergarments, pullovers, front-opening shirts and blouses, managing zippers, buttons, and snaps:

- [ ] **0 - Able to get clothes out of closets and drawers, put them on and remove them from the upper body without assistance.**
- [x] **1 - Able to dress upper body without assistance if clothing is laid out or handed to the patient.**
- [ ] **2 - Someone must help the patient put on upper body clothing.**
- [ ] **3 - Patient depends entirely upon another person to dress the upper body.**

(M1820)

Current **Ability to Dress Lower Body** safely (with or without dressing aids) including undergarments, slacks, socks or nylons, shoes:

- [ ] **0 - Able to obtain, put on, and remove clothing and shoes without assistance.**
- [ ] **1 - Able to dress lower body without assistance if clothing and shoes are laid out or handed to the patient.**
- [x] **2 - Someone must help the patient put on undergarments, slacks, socks or nylons, and shoes.**
- [ ] **3 - Patient depends entirely upon another person to dress lower body.**

(M1830) Bathing:

Current ability to wash entire body safely. **Excludes grooming (washing face, washing hands, and shampooing hair).**

- [ ] **0 - Able to bathe self in shower or tub independently, including getting in and out of tub/shower.**
- [ ] **1 - With the use of devices, is able to bathe self in shower or tub independently, including getting in and out of the tub/shower.**
- [ ] **2 - Able to bathe in shower or tub with the intermittent assistance of another person: (a) for intermittent supervision or encouragement or reminders, OR (b) to get in and out of the shower or tub, OR (c) for washing difficult to reach areas.**
- [ ] **3 - Able to participate in bathing self in shower or tub, but requires presence of another person throughout the bath for assistance or supervision.**
- [x] **4 - Unable to use the shower or tub, but able to bathe self independently with or without the use of devices at the sink, in chair, or on commode.**
- [ ] **5 - Unable to use the shower or tub, but able to participate in bathing self in bed, at the sink, in bedside chair, or on commode, with the assistance or supervision of another person**
- [ ] **6 - Unable to participate effectively in bathing and is bathed totally by another person.**

(M1840) Toilet Transferring:

Current ability to get to and from the toilet or bedside commode safely and transfer on and off toilet/commode

- ○ **0 - Able to get to and from the toilet and transfer independently with or without a device**
- ✓ **1 - When reminded, assisted, or supervised by another person, able to get to and from the toilet and transfer.**
- ○ **2 - Unable to get to and from the toilet but is able to use a bedside commode (with or without assistance).**
- ○ **3 - Unable to get to and from the toilet or bedside commode but is able to use a bedpan/urinal independently**
- ○ **4 - Is totally dependent in toileting.**

(M1845) Toileting Hygiene:

Current ability to maintain perineal hygiene safely, adjust clothes and/or incontinence pads before and after using toilet, commode, bedpan, urinal. If managing ostomy, includes cleaning area around stoma, but not managing equipment.

- ○ **0 - Able to manage toileting hygiene and clothing management without assistance.**
- ✓ **1 - Able to manage toileting hygiene and clothing management without assistance if supplies/implements are laid out for the patient.**
- ○ **2 - Someone must help the patient to maintain toileting hygiene and/or adjust clothing.**
- ○ **3 - Patient depends entirely upon another person to maintain toileting hygiene**

(M1850) Transferring:

Current ability to move safely from bed to chair, or ability to turn and position self in bed if patient is bedfast

- ○ **0 - Able to independently transfer.**
- ○ **1 - Able to transfer with minimal human assistance or with use of an assistive device.**
- ✓ **2 - Able to bear weight and pivot during the transfer process but unable to transfer self.**
- ○ **3 - Unable to transfer self and is unable to bear weight or pivot when transferred by another person**
- ○ **4 - Bedfast, unable to transfer but is able to turn and position self in bed.**
- ○ **5 - Bedfast, unable to transfer and is unable to turn and position self**

(M1860) Ambulation/Locomotion

Current ability to walk safely, once in a standing position, or use a wheelchair, once in a seated position, on a variety of surfaces.

- ○ **0 - Able to independently walk on even and uneven surfaces and negotiate stairs with or without railings (specifically: needs no human assistance or assistive device).**
- ○ **1 - With the use of a one-handed device (for example, cane, single crutch, hemi-walker), able to independently walk on even and uneven surfaces and negotiate stairs with or without railings.**
- ○ **2 - Requires use of a two-handed device (for example, walker or crutches) to walk alone on a level surface and/or requires human supervision or assistance to negotiate stairs or steps or uneven surfaces.**
- ○ **3 - Able to walk only with the supervision or assistance of another person at all times.**
- ✓ **4 - Chairfast, unable to ambulate but is able to wheel self independently.**
- ○ **5 - Chairfast, unable to ambulate and is unable to wheel self**
- ○ **6 - Bedfast, unable to ambulate or be up in a chair.**

GG0130 - SELF CARE

Code the patient's usual performance at SOC/ROC for each activity using the 6-point scale. If activity was not attempted at SOC/ROC, code the reason.
Coding:
Safety and Quality of Performance – If helper assistance is required because patient's performance is unsafe or of poor quality, score according to amount of assistance provided.
Activities may be completed with or without assistive devices.

06. Independent – Patient completes the activity by him/herself with no assistance from a helper.
05. Setup or clean-up assistance – Helper sets up or cleans up; patient completes activity. Helper assists only prior to or following the activity.
04. Supervision or touching assistance – Helper provides verbal cues and/or touching/steadying and/or contact guard assistance as patient completes activity. Assistance may be provided throughout the activity or intermittently.
03. Partial/moderate assistance – Helper does LESS THAN HALF the effort. Helper lifts, holds or supports trunk or limbs, but provides less than half the effort.
02. Substantial/maximal assistance – Helper does MORE THAN HALF the effort. Helper lifts or holds trunk or limbs and provides more than half the effort.
01. Dependent – Helper does ALL of the effort. Patient does none of the effort to complete the activity. Or, the assistance of 2 or more helpers is required for the patient to complete the activity.

If activity was not attempted, code reason:

07. Patient refused
09. Not applicable – Not attempted and the patient did not perform this activity prior to the current illness, exacerbation or injury.
10. Not attempted due to environmental limitations (e.g., lack of equipment, weather constraints)
88. Not attempted due to medical conditions or safety concerns

(GG0130)SOC/ROC Performance

A. Eating: The ability to use suitable utensils to bring food and/or liquid to the mouth and swallow food and/or liquid once the meal is placed before the patient.
B. Oral Hygiene: The ability to use suitable items to clean teeth. Dentures (if applicable): The ability to insert and remove dentures into and from mouth, and manage denture soaking and rinsing with use of equipment.
C. Toileting Hygiene: The ability to maintain perineal hygiene, adjust clothes before and after voiding or having a bowel movement. If managing an ostomy, include wiping the opening but not managing equipment.
E. Shower/bathe self: The ability to bathe self, including washing, rinsing, and drying self (excludes washing of back and hair). Does not include transferring in/out of tub/shower.
F. Upper body dressing: The ability to dress and undress above the waist; including fasteners, if applicable.
G. Lower body dressing: The ability to dress and undress below the waist, including fasteners; does not include footwear.
H. Putting on/taking off footwear: The ability to put on and take off socks and shoes or other footwear that is appropriate for safe mobility; including fasteners, if applicable.

	A. Eating	B. Oral Hygiene	C. Toileting Hygiene	E. Shower /bathe self	F. Upper body dressing	G. Lower body dressing	H. Putting on /taking off footwear
(06) Independent	○	○	○	○	○	○	○
(05) Setup or clean-up assistance	✓	✓	✓	○	✓	○	○
(04) Supervision or touching assistance	○	○	○	○	○	○	○
(03) Partial/moderate assistance	○	○	○	✓	○	✓	✓
(02) Substantial/maximal assistance	○	○	○	○	○	○	○
(01) Dependent	○	○	○	○	○	○	○
(07) Patient refused	○	○	○	○	○	○	○
(09) Not applicable	○	○	○	○	○	○	○
(10) Not attempted due to environmental limitations	○	○	○	○	○	○	○
(88) Not attempted due to medical conditions or safety concerns	○	○	○	○	○	○	○
(-) Not assessed/no information	○	○	○	○	○	○	○

DME and Supplies

DME Lookup

Durable Medical Equipment:
Wheelchair

#18B ACTIVITIES PERMITTED

- ○ **1- Complete bedrest**
- ✓ **5 - Exercises prescribed**
- ✓ **A - Wheelchair**

- ◯ 2 - Bedrest/BRP
- ☑ 3 - Up as tolerated
- ☑ 4 - Transfer bed/chair
- ◯ 6 - Partial weight bearing
- ◯ 7 - Independent in home
- ◯ 8 - Crutches
- ◯ 9 - Cane
- ◯ B - Walker
- ◯ C - No restrictions
- ☑ D - Other (specify)

Assist with ADLs

DIAGNOSIS

M1100. Patient Living Situation. Which of the following best describes the patient's residential circumstance and availability of assistance?

Living Arrangement	Around the clock	Regular daytime	Regular Night time	Short-term Assistance	No Assistance available
A. Patient lives alone	◯	◯	◯	✓	◯
B. Patient lives with other person(s) in the home	◯	◯	◯	◯	◯
C. Patient lives in congregate situation (for example, assisted living, residential care home)	◯	◯	◯	◯	◯

Primary Caregiver
Polly Johnson

Relationship to Patient
Friend

Contact Number
702-575-0727

Comments
Patient lives alone with occasional help from her friend

Name Of Facility

Phone:

Able to safely care for patient?
☑ Yes ◯ No

Home Environment / Home Safety Assessment

- ☐ Architectural Barriers
- ☑ Medications are improperly stored
- ☐ No telephone
- ☐ Cluttered surroundings
- ☐ Inadequate railing
- ☐ Unsafe oxygen use
- ☐ No fire extinguisher
- ☐ Inadequate electricity / cooling / heating
- ☐ Inadequate supply of water
- ☐ Inadequate sanitation / plumbing
- ☐ Inadequate smoke detectors on all levels of home
- ☐ Unsafe storage of equipments
- ☐ No emergency exit plan
- ☐ Rodent / insect infestation
- ☐ Unsafe electrical wiring
- ☐ Nonworking stove / refrigerator
- ☐ Exposed flammables
- ☐ Inadequate Lighting

Other:

Type of Assistance Patient Receives - other than from Home Health Agency Staff:

ADL(Bathing, dressing, toileting, eating/feeding, bowel)	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
IADL (Meds, meals, house-keeping, laundry, telephone, shipping, finances)	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Psychosocial Support	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Assistance with doctors appointment, delivery of medications	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations
Management of Finances	☑ Family/Friends	☐ Provider Services	☐ Paid Caregiver	☐ Volunteer Organizations

Comments:

Safety Hazards Affecting Patient:

- ☐ Stairs
- ☐ Inadequate lighting, heating and cooling
- ☐ Insect/Rodent Infestation
- ☐ No gas/electric appliance
- ☐ No running water, plumbing
- ☐ Narrow or obstructed walkway
- ☐ Lack of fire safety devices
- ☐ Cluttered/soiled living area
- ☑ No hazards identified

Others:

(M2102) Types and Sources of Assistance:

Determine the ability and willingness of non-agency caregivers (such as family members, friends, or privately paid caregivers) to provide assistance for the following activities, if assistance is needed. Excludes all care by your agency staff

F. Supervision and Safety (for example, due to cognitive impairment)

- ◯ No assistance needed –patient is independent or does not have needs in this area
- ◯ Non-agency caregiver(s) currently provide assistance
- ☑ Non-agency caregiver(s) need training/ supportive services to provide assistance
- ◯ Non-agency caregiver(s) are not likely to provide assistance OR it is unclear if they will provide assistance
- ◯ Assistance needed, but no non-agency caregiver(s) available

Safety Measures

- ◯ 1 - Bleeding precautions
- ☑ 2 - O_2 precautions
- ◯ 3 - Seizure precautions
- ☑ 4 - Fall precautions
- ◯ 5 - Aspiration precautions
- ◯ 6 - Siderails up
- ◯ 7 - Elevate head of bed
- ◯ 8 - 24 hr. supervision
- ☑ 9- Clear Pathways up
- ☑ 10 - Lock w/c with transfers
- ☑ 11 - Infection control measures
- ◯ 12 - Walker/cane
- ◯ 13 - Sharps Precaution
- ☑ 14 - Other:

ER/911 protocol, COVID-19, and universal precautions

(M1021/1023) Diagnoses and Symptom Control

List each diagnosis for which the patient is receiving home care in Column 1, and enter its ICD-10-CM code at the level of highest specificity in Column 2 (diagnosis codes only – no surgical or procedure codes allowed). Diagnoses are listed in the order that best reflects the seriousness of each condition and supports the disciplines and services provided. Rate the degree of symptom control for each condition in Column 2. ICD-10-CM sequencing requirements must be followed if multiple coding is indicated for any diagnoses.

Code each row according to the following directions for each column:

- **Column** 1: Enter the description of the diagnosis. Sequencing of diagnoses should reflect the seriousness of each condition and support the disciplines and services provided. Column 2: Enter the ICD-10-CM code for the condition described in Column 1 – no surgical or procedure codes allowed. Codes must be entered at the level of highest specificity and ICD-10-CM coding rules and sequencing requirements must be followed. Note that external cause codes (ICD-10-CM codes beginning with V, W, X, or Y) may not be reported in M1021 (Primary Diagnosis) but may be reported in M1023 (Secondary Diagnoses). Also note that when a Z-code is reported in

- **Column 2 :** the code for the underlying condition can often be entered in Column 2, as long as it is an active on-going condition impacting home health care. Rate the degree of symptom control for the condition listed in Column 1. Do not assign a symptom control rating if the diagnosis code is a V, W, X, Y or Z-code. Choose one value that represents the degree of symptom control appropriate for each diagnosis using the following scale: 0 - Asymptomatic, no treatment needed at this time 1 - Symptoms well controlled with current therapy 2 - Symptoms controlled with

difficulty, affecting daily functioning; patient needs ongoing monitoring 3 - Symptoms poorly controlled; patient needs frequent adjustment in treatment and dose monitoring 4 - Symptoms poorly controlled; history of re-hospitalizations Note that the rating for symptom control in Column 2 should not be used to determine the sequencing of the diagnoses listed in Column 1. These are separate items and sequencing may not coincide.

	Onset Date	Severity	
PRIMARY DIAGNOSIS M17.0 - Bilateral primary osteoarthritis of knee	03/30/2026	0 1 2 **3** 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 1 I89.0 - Lymphedema, not elsewhere classified	03/30/2026	0 1 2 **3** 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 2 I12.9 - Hypertensive chronic kidney disease w stg 1-4/unsp chr kdny	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 3 N18.31 - Chronic kidney disease, stage 3a	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 4 J44.9 - Chronic obstructive pulmonary disease, unspecified	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
OTHER DIAGNOSIS - 5 J96.11 - Chronic respiratory failure with hypoxia	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
Z99.81 - Dependence on supplemental oxygen	03/30/2026	0 1 2 3 4 **^**	**Onset** Exacerbate
I48.0 - Paroxysmal atrial fibrillation	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
I70.0 - Atherosclerosis of aorta	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
I49.5 - Sick sinus syndrome	03/30/2026	0 1 **2** 3 4 ^	Onset **Exacerbate**
I87.2 - Venous insufficiency (chronic) (peripheral)	03/30/2026	0 1 **2** 3 4 ^	

			○ Onset ◉ Exacerbate
E66.01 - Morbid (severe) obesity due to excess calories	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^
			○ Onset ◉ Exacerbate
Z68.42 - Body mass index [BMI] 45.0-49.9, adult	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^
			○ Onset ◉ Exacerbate
N25.81 - Secondary hyperparathyroidism of renal origin	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^
			○ Onset ◉ Exacerbate
D69.2 - Other nonthrombocytopenic purpura	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^
			○ Onset ◉ Exacerbate
R73.03 - Prediabetes	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^
			○ Onset ◉ Exacerbate
L84. - Corns and callosities	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4 ○ ^
			○ Onset ◉ Exacerbate
Z95.0 - Presence of cardiac pacemaker	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^
			◉ Onset ○ Exacerbate
Z74.09 - Other reduced mobility	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^
			◉ Onset ○ Exacerbate
Z91.81 - History of falling	**Onset Date** 03/30/2026	**Severity**	○ 0 ○ 1 ○ 2 ○ 3 ○ 4 ◉ ^
			◉ Onset ○ Exacerbate

(M1028) Active Diagnoses – Comorbidities and Co-existing Conditions – Check all that apply See OASIS Guidance Manual for a complete list of relevant ICD-10 codes.

- ☐ Diabetes Mellitus (DM)
- ☐ Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)
- ☑ None of the above

(M1033) Risk for Hospitalization: Which of the following signs or symptoms characterize this patient as at risk for hospitalization? (Mark all that apply.)

- ☐ 1 - History of falls (2 or more falls – or any fall with an injury – in the past 12 months)
- ☐ 2 - Unintentional weight loss of a total of 10 pounds or more in the past 12 months
- ☐ 3 - Multiple hospitalizations (2 or more) in the past 6 months
- ☑ 6 - Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months
- ☑ 7 - Currently taking 5 or more medications
- ☑ 8 - Currently reports exhaustion

- [ ] 4 - Multiple emergency department visits (2 or more) in the past 6 months
- [ ] 5 - Decline in mental, emotional, or behavioral status in the past 3 months
- [x] 9 - Other risk(s) not listed in 1 - 8
- [ ] 10 - None of the above

Additional Risk Information:
Pt at risk for hospitalization due to multiple diagnoses/comorbidities.
Pt at risk for currently taking 3 or more medications
Pt at risk for unstable blood pressure complications r/t HTN
Pt at risk for injury and falls r/t ambulation difficulties from weakness
Pt at risk for fluid volume excess r/t CKD
Pt is at risk for Cardiovascular complications r/t Atrial fibrillation
Pt at risk for respiratory distress r/t COPD

Prior Hospitalization

- ( ) Yes
- (x) No

Number of times

Hospitalization Risk Assessment

Prior Pattern: Check all that apply

- [ ] > 1 Hospitalizations or ER visits in the past 12 months
- [x] History of falls

Chronic conditions: Check all that apply (M1020/1022/1024)

- [ ] HF
- [ ] Diabetes
- [x] COPD
- [ ] Chronic skin ulcers (Wound consult if indicated for any wounds)
- [ ] HIV/AIDS

Risk factors: (Check all that apply)

- [ ] Discharged from hospital or skilled nursing facility
- [x] More than 2 secondary diagnoses
- [ ] Low socioeconomic status or financial concerns ♦
- [x] Lives alone ►♦
- [x] Inadequate support network ♦
- [x] ADL assistance needed ►
- [x] Home safety risks ► ♦
- [x] Dyspnea ► ★
- [x] Help with managing medications needed ► ★
- [x] Non-compliance with medication regimen ♦ ★
- [ ] Confusion ♦ ★
- [ ] Pressure ulcer ★
- [ ] Stasis ulcer ★
- [ ] Overall Poor Status/Prognosis ■
- [ ] Low literacy level ♦
- [ ] Depression ♦

- [x] ►Consider Therapy referral (PT, OT, ST)
- [x] ♦Consider MSW referral
- [x] ■ Consider Hospice referral
- [x] ★Consider RN referral, if not ordered

Total number of checked boxes is
14

Your agency may want to select a threshold score to target patients at high risk.

(For example: 5 or greater risk factors may indicate that the patient is at risk for hospitalization. Note: This number is for convenience only and has not been tested or validated. The agency may modify the score based upon the needs of their patient population.)

Carry out patient specific interventions as appropriate/ordered, if patient is at risk for hospitalization: (Coordinate with M2250)

Referrals:

- [x] SN
- [x] PT
- [ ] OT
- [ ] ST
- [ ] MSW
- [ ] HHA
- [ ] Dietary Consultant

Other

Consider implementing any of the following interventions, if patient is at risk for hospitalization:

- [x] Medication Management
- [x] Medication Reconciliation
- [x] Assess patient's: knowledge, ability, resources and adherence
- [x] Education
- [x] Patient/family education
- [ ] Enrollment into a disease management program

(Specify)

- [ ] Hospice/Palliative Referral
- [x] Individualized Patient Emergency Care Plan
- [x] Fall Prevention Program
- [ ] Phone Monitoring
- [ ] Telemonitoring
- [ ] Wound Care
- [ ] Front-loading Visits
- [ ] Care Coordination (physicians, hospitals, nursing homes ...)

Immunizations (M1040, M1045, M1050, M1055)

- [ ] Influenza
- [ ] Pneumococcal

Fall Prevention Program
Discussed fall-preventative measures: slow position changes, asking for help as needed, maintaining adequate lighting, keeping the pathway clear and clutter-free, and using an assistive device for safety.
Other

MAHC 10* - FALL RISK ASSESSMENT TOOL

	Yes	No
Age 65+	✓	
Diagnosis (3 or more co-existing) Includes only documented medical diagnosis	✓	
Prior history of falls within 3 months An unintentional change in position resulting in coming to rest on the ground or at a lower level		✓
Incontinence Inability to make it to the bathroom or commode in timely manner. Includes frequency, urgency, and/or nocturia.	✓	
Visual impairment Includes but not limited to, macular degeneration, diabetic retinopathies, visual field loss, age related changes, decline in visual acuity, accommodation, glare tolerance, depth perception, and night vision or not wearing prescribed glasses or having the correct prescription.	✓	
Impaired functional mobility May include patients who need help with IADLS or ADLS or have gait or transfer problems, arthritis, pain, fear of falling, foot problems, impaired sensation, impaired coordination or improper use of assistive devices.	✓	
Environmental hazards May include but not limited to, poor illumination, equipment tubing, inappropriate footwear, pets, hard to reach items, floor surfaces that are uneven or cluttered, or outdoor entry and exits.		✓

Poly Pharmacy (4 or more prescriptions - any type) All PRESCRIPTIONS including prescriptions for OTC meds. Drugs highly associated with fall risk include but not limited to, sedatives, antidepressants, tranquilizers, narcotics, antihypertensives, cardiac meds, corticosteroids, anti-anxiety drugs, anticholinergic drugs, and hypoglycemic drugs.	☑	☐
Pain affecting level of function Pain often affects an individual's desire or ability to move or pain can be a factor in depression or compliance with safety recommendations.	☑	☐
Cognitive impairment Could include patients with dementia, Alzheimer's or stroke patients or patients who are confused, use poor judgment, have decreased comprehension, impulsivity, memory deficits. Consider patients ability to adhere to the plan of care.	☐	☑
A score of 4 or more is considered at risk for falling	7	

MEDICATION AND ALLERGIES

(N0415) High-Risk Drug Classes: Use and Indication:

	1. Is Taking 1. Is taking Check if the patient is taking any medications by pharmacological classification, not how it is used, in the following classes	2. Indication Noted 2. Indication noted If Column 1 is checked, check if there is an indication noted for all medications in the drug class
A. Antipsychotic	☐	☐
E. Anticoagulant	☐	☐
F. Antibiotic	☐	☐
H. Opioid	☐	☐
I. Antiplatelet	☐	☐
J. Hypoglycemic (including insulin)	☐	☐
Z. None of the Above	☑	

Medication

☐	Start Date	Medication	Strength / Dosage / Frequency	Route	Classification/Indication	Status
☐	03/30/2026	Gabapentin (Oral Pill)	300 mg 1 Cap Three times a day	By mouth	ANTICONVULSANTS /	New
☐	03/30/2026	Lisinopril (Oral Pill)	40 mg 1 Tab Once a day	By mouth	ACE INHIBITORS /	New
☐	03/30/2026	hydrochlorothiazide	25 mg 1 Tab Once a day	By mouth	THIAZIDE DIURETICS /	New
☐	03/30/2026	Multivitamins	1 Tab Once a day	By mouth	SUPPLEMENT /	New
☐	03/30/2026	DIALYVITE VITAMIN D3 MAX	1.25 mg 1 Tab Once a day	By mouth	VITAMIN D,OTHER /	New
☐	03/30/2026	Vitamin B 12	1 Tab Once a day	By mouth	Vitamin B 12 /	New
☐	03/30/2026	Acetaminophen (Oral Pill)	500 mg 1 Tab As needed	By mouth	NON-OPIOID ANALGESICS / for mid to moderate pain	New

Medication Remarks
Medication remarks...

☐ Discontinued Medications

Start Date	Medication	Strength / Dose / Frequency / Route	Classification/Indication	Laboratory	Status	Disc Date	Remarks

Does patient have IV access? ○ **Yes** ● **No**

(M2001) Drug Regimen Review

Did a complete drug regimen review identify potential clinically significant medication issues?

- ○ **No – No issues found during review [GO TO M2010]**
- ● **Yes – Issues found during review**
- ○ **Not assessed/no information**
- ○ **NA – Patient is not taking any medications [GO TO M2102]**

(M2003) Medication Follow-up

Did the agency contact a physician (or physician-designee) by midnight of the next calendar day and complete prescribed/recommended actions in response to the identified potential clinically significant medication issues?

- ○ **No**
- ● **Yes**
- ○ **Not assessed/no information**

M2010. Patient/Caregiver High-Risk Drug Education

Has the patient/caregiver received instruction on special precautions for all high-risk medications (such as hypoglycemics, anticoagulants, etc.) and how and when to report problems that may occur?

- ○ **NO**
- ○ **YES**
- ● **NA - Patient not taking any high-risk drugs OR patient/caregiver fully knowledgeable about special precautions associated with all high-risk medications**

M2020. Management of Oral Medications

Patient's current ability to prepare and take all oral medications reliably and safely, including administration of the correct dosage at the appropriate times/intervals. Excludes injectable and IV medications. (NOTE: This refers to ability, not compliance or willingness.)

- ○ **Able to independently take the correct oral medication(s) and proper dosage(s) at the correct times.**
- ● **Able to take medication(s) at the correct times if: (a) individual dosages are prepared in advance by another person; OR (b) another person develops a drug diary or chart.**
- ○ **Able to take medication(s) at the correct times if given reminders by another person at the appropriate times.**
- ○ **Unable to take medication unless administered by another person**
- ○ **NA - No oral medications prescribed**

M2030. Management of Injectable Medications

Patient's current ability to prepare and take all prescribed injectable medications reliably and safely, including administration of correct dosage at the appropriate times/intervals. Excludes IV medications.

- ◯ **Able to independently take the correct medication(s) and proper dosage(s) at the correct times**
- ◯ **Able to take injectable medication(s) at the correct times if: a. individual syringes are prepared in advance by another person; OR b. another person develops a drug diary or chart.**
- ◯ **Able to take medication(s) at the correct times if given reminders by another person based on the frequency of the injection**
- ◯ **Unable to take injectable medication unless administered by another person**
- ● **NA - No injectable medications prescribed.**

Allergies

Name	Reaction	Start Date
		None known

Add Allergies

(O0110). Special Treatments, Procedures, and Programs

Check all of the following treatments, procedures, and programs that apply at discharge.

	On Admission (Check All That Apply)
Cancer Treatment	
A1. Chemotherapy	☐
A2. IV	■
A3. Oral	■
A10.Other	■
B1. Radiation	☐
Respiratory Therapies	
C1. Oxygen Therapy	☑
C2. Continuous	☑
C3. Intermittent	☐
C4. High-Concentration	☐
D1. Suctioning	☐
D2. Scheduled	■
D3. As needed	■
E1. Tracheostomy Care	☐
F1. Invasive Mechanical Ventilator	☐
G1. Non-invasive Mech. Vent.	☐
G2. BiPAP	■
G3. CPAP	■
Others	
H1. IV Medications	☐
H2. Vasoactive medications	■
H3. Antibiotics	■
H4. Anticoagulation	■
H10.Other	■
I1. Transfusions	☐
J1. Dialysis	☐
J2. Hemodialysis	■
J3. Peritoneal dialysis	■
O1. IV Access	☐
O2. Peripheral	■
O3. Mid-line	■
O4. Central (e.g., PICC, tunneled, port)	■
Z1. None of the Above	☐

PATIENT COORDINATION AND CARE PLAN

Plan of Care Orders

Orders and Treatment (Specify Frequency, Duration and Purpose of Visits): §484.60 (2.iv)

SN Frequency: 2/wk x wk1, 1/wk x wk3

MSW Frequency:

PT Frequency: For evaluation and treatment

OT Frequency:

HHA Frequency:

ST Frequency

Dietitian Frequency:

*****MD SIGNATURE COVERS ALL PAGES*****
ADMIT PATIENT TO AVERY HOME HEALTH SERVICES. ALL DIAGNOSIS AND MEDICATIONS VERIFIED WITH THE PRIMARY CARE PHYSICIAN.

REASON FOR HOMEBOUND:
The patient is considered homebound because she is an 80-year-old female with severe functional and medical impairments that prevent safe, independent leaving of the home without assistance. Objective findings from the record document morbid obesity (BMI 52.4), bilateral primary osteoarthritis of the knees with marked functional decline, bilateral lower-extremity lymphedema/venous stasis with dependent edema, painful bilateral foot calluses, chronic hypoxic respiratory failure/COPD (on supplemental oxygen as needed), and chronic kidney disease stage 3. Functionally she uses a walker and reports she is only able to ambulate a few steps, is unable to stand for more than a few seconds, becomes short of breath and experiences exertional lower-extremity pain with ambulation, and requires a power chair for intra-home mobility (current power chair is too large for safe use at home and she is in the process of obtaining an appropriately sized chair). She is able to transfer but has very limited ability to ambulate and is at high fall risk due to extreme obesity, knee osteoarthritis, lymphedema, neuropathic pain, and deconditioning. The patient lives alone with no live-in caregiver (friend provides near-daily assistance), which increases safety risk if she were to leave home and require unassisted ambulation or recovery from exertion. Leaving the home requires a considerable and taxing effort because even minimal ambulation provokes dyspnea and lower-extremity pain, she can only take a few steps and cannot sustain standing long enough to perform transfers or traverse typical community distances, and her current mobility equipment is inadequate for safe community ambulation. Exiting the home would therefore require either multiple persons and/or specialized equipment (appropriately sized power chair and transport) and poses significant fall and respiratory risk. Allowable, limited absences are restricted to medically necessary appointments or brief tolerated activities with appropriate transport and assistance; prolonged or frequent community outings would exceed her functional tolerance and increase risk of decompensation. Given these documented impairments (morbid obesity, bilateral knee osteoarthritis, bilateral lower-extremity lymphedema with dependent edema, chronic hypoxic respiratory failure/COPD, CKD stage 3, painful foot calluses, inability to ambulate more than a few steps, requirement for power mobility, and lack of a live-in caregiver), skilled home health services are medically necessary to provide skilled nursing for medication adjustment and renal monitoring after diuretic change, supervised management of oxygen therapy and respiratory symptoms, and skilled PT/OT for strengthening, gait/transfer training, power-chair assessment and training, and fall-risk mitigation, as well as skilled wound/podiatry coordination for foot calluses—services required to restore or maintain function and to safely manage her chronic medical conditions at home.

FREQUENCY AND DURATION:
HH SN frequency: 2 times a week for 1 week, 1 time a week for 3 weeks for skilled observation and assessment, medication reconciliation, pain assessment, wound monitoring and management, infection control measures, diet, home safety and disease process education.
HH PT frequency: For evaluation and treatment for mobility, gait, transfer, establish therapeutic exercises, strengthening, home safety and fall prevention techniques training.

OXYGEN SUPPLEMENT ORDER:
Oxygen Therapy Order: Continuous Oxygen at 3LPM via nasal cannula for shortness of breath.

GOALS:
-Patient will demonstrate safe use of walker and power chair with fewer verbal cues, showing improved transfer ability and reduced fall risk within 4 weeks.
-Patient will maintain oxygen saturation ≥ 92% at rest and ≥ 90% with activity, as evidenced by skilled nursing monitoring and documentation within 4 weeks.
-Patient will verbalize understanding of diuretic regimen and renal monitoring needs, with adherence to fluid restriction and recognition of signs of imbalance within 4 weeks.
-Patient will report improved pain control in knees and feet, with pain reduced by at least 2 points on the pain scale during mobility or ADLs within 4 weeks.
-Patient will remain free from falls or falls-related injuries in the home, demonstrating increased safety awareness and consistent use of assistive devices within 4 weeks.

Patient Strengths

- [x] Motivated Learner
- [x] Strong Support System
- [ ] Absence of Multiple Diagnosis
- [ ] Enhanced Socioeconomic Status

Other:

Conclusions

- [x] Skilled Intervention Needed
- [x] Skilled Instruction Needed
- [ ] No Skilled Service Needed

Other:

Patient Summary / Clinical Narrative

Elizabeth Root is an 80-year-old female patient, referred for home health care on March 26, 2026. The attending physician overseeing her care is Dr. Stacy Leifried. The assessment for home health services was completed by Emiliya Berberova, RN, on March 30, 2026. It is noteworthy that Elizabeth was not discharged from any inpatient facility within the past 14 days, indicating a direct referral for home health services without prior hospitalization.

Elizabeth is classified as homebound due to multiple factors. Her condition prevents her from leaving the home without significant effort. She experiences difficulty standing or walking for extended periods and requires human assistance due to an unsteady gait and poor balance. Additionally, she finds leaving the home exhausting, often necessitating extended rest periods to recover from any exertion.

The primary reason for Elizabeth's need for home health services is her diagnosis of bilateral primary osteoarthritis of the knees (diagnostic code M17.0), which significantly impacts her mobility and overall functional ability. In addition to this primary diagnosis, she has comorbidities that include lymphedema, chronic kidney disease, chronic obstructive pulmonary disease (COPD), and chronic respiratory failure with hypoxia. These conditions further complicate her health status and increase her need for skilled services.

A comprehensive head-to-toe assessment reveals several key findings. Cardiopulmonary evaluation indicates a heart rate of 64 beats per minute, a blood pressure of 149/78 mmHg, and a respiratory rate of 16 breaths per minute, which suggests moderate cardiovascular and respiratory function but with some concerns due to her COPD and chronic respiratory issues. Gastrointestinal assessment is unremarkable, but Elizabeth has reported occasional bowel incontinence. Integumentary assessment shows no acute skin issues, and safety assessments indicate a need for assistance with activities of daily living due to her mobility limitations.

Psychosocially, Elizabeth lives alone and demonstrates no signs of depression or anxiety, as evidenced by her PHQ-2 score of 0. She appears alert and oriented, with adequate cognitive function, and is able to engage in social interactions without notable difficulties. Her living arrangements are suitable, although her independence is limited by her physical capabilities.

Elizabeth's vital signs are stable, with a temperature of 96.9°F, which is within normal limits. Her cognitive function has been assessed through the Brief Interview for Mental Status (BIMS), scoring 15, indicating no acute changes in her mental status. She remains oriented to person, place, time, and situation, with coherent thought processes and appropriate behavior during the assessment.

In terms of activities of daily living (ADLs), Elizabeth requires significant assistance. She can dress her upper body independently if clothing is prepared for her but needs help with lower body dressing and transfers. She is chair-bound and cannot ambulate independently, although she can wheel herself with the use of a wheelchair. Her functional ability is classified as independent in self-care but needing assistance for mobility and ADLs.

Elizabeth's mobility is severely limited, with reports of dyspnea upon moderate exertion, such as during dressing or transferring. She utilizes a walker for support and requires assistance with transfers, indicating a need for skilled nursing interventions to help manage her mobility issues safely.

Socially, Elizabeth has reported limited access to transportation, which affects her ability to attend medical appointments and obtain necessary medications. This lack of support necessitates regular home health visits to ensure her medical needs are met.

A thorough review of Elizabeth's medications reveals that she is currently taking five or more medications daily, including those for hypertension, diabetes, and her chronic conditions. Attention needs to be given to potential drug interactions and adherence to her medication regimen due to her reported history of difficulty complying with medical instructions.

Elizabeth's primary caregiver is not formally identified, but she does have informal support from neighbors and friends who help as needed. However, these supports may require training and assistance to effectively provide care.

In conclusion, Elizabeth Root's medical necessity for skilled home health services is clear, given her primary diagnosis of bilateral osteoarthritis and multiple comorbidities that affect her mobility and overall health. The need for skilled nursing visits is critical to manage her medications, assist with ADLs, and monitor her chronic conditions. A visit frequency of two to three times per week is recommended to ensure comprehensive management of her care and to address her evolving health needs.

Skilled Intervention

Assessment/Instruction/Performance:
Explained Home Health service to patient/family/caregiver(s). Explained consent to care, release of liability, and patient rights & responsibilities. Consent to care, a liability release, and consent to photograph were signed by the patient/responsible party. A comprehensive assessment on all body systems with OASIS elements evaluation was initiated. Reconciled facility discharges medications against routine home prescriptions without issue. Discussed advance directives, privacy policy, COVID-19 prevention, 911 protocols, basic home safety, and fall prevention principles with patient/family/caregiver(s). Discussed plan of care draft with patient and explained what to expect in the next visits and the remainder of the care episode. Also explained that the initial discussion on the management of other disease processes will be done on the next visit.

Assessed patient's musculoskeletal status due to Osteoarthritis. Instructed patient for signs and symptoms such joint stiffness, swelling, pain which worsens with activities and is not relieved with rest, progressive loss in the range of motion, crepitation, increasing weakness, fatigue, and increasing morning stiffness.

Instructed patient/caregiver on measures to alleviate edema such as elevating BLEs above the level of the heart while lying down or on a stool/chair while sitting.

Instructed patient to adhere and take pain medication before pain becomes severe to achieve pain control. Provided teachings on nonpharmacologic pain relief measures such as relaxation techniques and positioning. Instructed patient/PCG to notify SN/MD if patient is experiencing pain level greater than 7/10, unable to tolerate pain, and if pain is affecting ability to perform normal activities/ADLs.

Assessed pt's safety in environment, mobility and functional limitations to reduce falls or accidents every visit. Assessed the patient's ability to perform ADLs effectively and safely every visit. Instructed Patient/Caregiver on the importance of assessing conditions/risks that may contribute to occurrence of falls such as fatigue/ muscle weakness. Encouraged to perform exercises if tolerated for strengthening and maintenance of posture and flexibility. Established HEP and educate Patient/Caregiver on passive/active assistive ROM exercises and encouraged to perform exercises to maintain muscle strength and endurance.

Reinforced patient/caregiver on fall prevention techniques including use of proper assistive device, locking wheelchair before transfers, and placing patient's items within arm's reach. Stress to patient to avoid attempting tasks that are too difficult for present level of functioning and ask for help when needed. Instructed the patient/caregiver to contact agency to report any fall with or without minor injury and to call 911 for fall resulting in serious injury or causing severe pain or immobility. Instructed on safety measures to prevent falls: remove clutter from walkways. Secure loose rugs with double-faced tape, tacks or a slip-resistant backing or remove loose rugs from home. Immediately clean spilled liquids, grease or food. Use nonslip mats inside bathtub or shower. Light up living space. Keep home brightly lit to avoid tripping on objects that are hard to see. Place night lights in the bedroom, bathroom and hallways. Place a lamp within reach of the bed for middle-of-the-night needs. Store flashlights in easy-to-find places in case of power outages. Use assistive devices such as walker, grab bars, commode. Wear properly fitting, sturdy shoes with nonskid soles.

Medication reconciliation was done during the visit. Patient/caregiver verbalized understanding in all teachings during the visit.

☑ **Response to Teaching/Interventions**

Verbalized Understanding	☑ **Patient (%):** 60%	☐ **Caregiver (%):**
Return Demonstration	☑ **Patient (%):** 60%	☐ **Caregiver (%):**
Require Further Teaching	☑ **Patient**	☑ **Caregiver**

Comments:
Patient verbalized 60% of understanding on the disease process taught during this visit. Patient unable to teach back complete understanding on fall precautions and hypertension urgency symptoms. Patient also redemonstrated 60% of home safety and fall prevention measures using assistive device wheelchair as patient still demonstrates weakness, poor endurance and ambulation difficulty. Patient and caregiver require further teaching.

Conference With:

☑ **MD**	☑ **SN**	☑ **PT**	☐ **OT**
☐ **ST**	☐ **MSW**	☐ **HHA**	☐ **DON/DOPCS**
☑ **Clinical Coordinator**	☑ **Clinical Manager**		

Name:
Nariah Hinds, RN CM
Regarding:
Start of Care, POC, visit frequencies, and update of the patient's health condition
Physician Contacted Re:
Start of Care, POC, visit frequencies, and update of the patient's health condition
Order Changes:
Plan for Next Visit:
Continue with skilled assessment and observation to monitor the patient's progress and response to treatment. Reinforce prior teachings on diet, medications, fall and safety precautions, and disease management
Next Physician Visit:
mm/dd/yyyy

Patient and caregiver education and training to facilitate timely discharge: §484.60 (2.xiii)
Medication management, disease process and management, skin care and prevention of pressure sores, diet, good nutrition/hydration, fall and home safety precautions, infection control, and COVID-19 precautions and prevention
Visit Narrative
Patient is an 80-year-old homebound female admitted to Avery Home Health Care. Patient has a known medical history of CKD stage 3, Hypertension, COPD, Chronic respiratory failure with hypoxia, Atrial fibrillation, Hyperparathyroidism, Osteoarthritis of both knees, Chronic back pain, Lymphedema of both lower extremities, Morbid obesity, Left shoulder dislocation, Sleep apnea. The patient lives alone with occasional help from her friend for assistance at home and during MD appointments. Patient complains of bilateral knee pain, interfering with functional status and impacting activities of daily living. Patient has an unsteady gait, poor balance, and generalized weakness, placing her at risk for falls and injury. Patient is alert, oriented, able to make needs known, able to follow instructions, cooperative, and verbally responsive. Patient has shortness of breath with moderate exertion. Skin is intact; no breakdown or impairment observed. Patient with severe lymphedema on both lower extremities; no open areas were noted. Podiatry referral was made. Patient is homebound due to current health condition, unsafe to leave home, requires assistance to complete ADLs/IADLs safely, and has a medical necessity to be under home health care to prevent potential complications of medical conditions. Patient currently relies on a motorized wheelchair for ambulation and transfer and requires assistance/supervision in ADL's/IADL's. A cane and walker do not meet her needs as it does not provide enough support, and the patient is unable to ambulate more than a few steps to transfer, making her a fall risk. She can stand from a seated position without assistance and get on her current power chair. She gets short of breath with ambulation and can only ambulate a few steps due to exertion as well as lower extremity pain. The patient exhibits functional deficits as evidenced by generalized weakness, low endurance, low activity tolerance, poor balance, and an unsteady gait, and requires assistive devices for safe mobility. Patient demonstrates a knowledge deficit related to disease processes and management, as well as medication management and compliance. Patient will benefit from home health care services for observation and assessment of the patient's response to treatment, medication reconciliation, pain assessment, and training and education on infection control measures, diet, fall and safety precautions, disease process, medications, and changes in clinical signs and symptoms. Skilled Nursing visits for assessment and monitoring of vital signs, medication and diet compliance, pain status, and other symptoms related to patients' diagnosis.
Physical Therapy for evaluation and assessment of rehab needs and to establish a treatment plan and implement a home exercise program.

Discussed consent for services, plan of care, patient's rights/responsibilities, HIPAA policies, and Home Health agency numbers were provided. Patient verbalized understanding, and consent was signed. Physical assessment, history taking, and vital signs were monitored. All home medications were reviewed and discussed, including indication, dose, frequency, schedule, possible side effects, and precautions. SN educated the patient on safety with ADLs, transfers, mobility, fall precautions, environmental safety, emergency plans, pain management, infection control measures, skin integrity, and energy-conserving measures. SN also educated the patient on signs and symptoms to report to the MD/SN, as well as signs and symptoms requiring an immediate ER visit or calling 911. SN followed COVID-19 infection precaution guidelines for healthcare professionals. Patient instructed on CDC-provided COVID-19 prevention measures. Patient denies symptoms of SOB, cough, high fever, or sore throat at this visit or in the last 48 hours. Patient verbalized understanding of all teachings.

Interventions/Instructions during visit:

☑ **Assess/Observe**	☐ **Injection Administration**	☐ **Catheter change**	☐ **IV Therapy**
☐ **Wound Care**	☑ **Teaching/Instructions**		

Discharge Plans

Discharge patient to self care.
Discharge when caregiver willing and able to manage all aspects of patient's care.
Discharge to care of physician.
Able to stay in residence with primary caregiver's assistance or community agency's support.
Discharge when medical condition is stable and patient is no longer in need of skilled services.
Discharge to caregiver.
Able to understand medication regime and care related to diagnoses.
Able to return to an independent level of care.
When goals are met.
Discharge when patient is independent in management of medical needs.
Discharge when goals met. Discharge when wound(s) healed.
Discharge when reliable caregiver available to assist with patient's medical needs.
When patient knows when to notify physician.

☐ **Written notice of discharge provided to patient**

Discharge scheduled for:
mm/dd/yyyy

Rehabilitation Potential

○ **Rehabilitation potential is poor**	○ **Rehabilitation potential is good**
● **Rehabilitation potential is fair**	○ **Rehabilitation potential is excellent**

Others:

OTHER SUPPLEMENTALS

Endocrine

- [ ] WNL (Within Normal Limits)

Is patient diabetic?	○ Yes	● No	
Insulin dependent?	○ Yes	○ No	How Long?
Is patient independently able to draw up correct dose of insulin?	○ Yes	○ No	○ N/A
Is patient able to administer own insulin?	○ Yes	○ No	○ N/A
Is patient taking oral hypoglycemic agent?	○ Yes	○ No	○ N/A
Is patient independent with glucometer use?	○ Yes	○ No	○ N/A
Is caregiver able to draw up and administer insulin?	○ Yes	○ No	○ N/A
Is caregiver independent with glucometer use?	○ Yes	○ No	○ N/A
Does patient or caregiver routinely perform inspection of the patient's lower extremities?	○ Yes	○ No	○ N/A

Does patient have any of the following?

- [ ] Polyuria
- [ ] Polydipsia
- [ ] Polyphagia
- [ ] Neuropathy
- [ ] Radiculopathy
- [x] Thyroid Problems

Comments

Past Medical History (Mark all that apply)

- [ ] CHF
- [ ] Cardiomyopathy
- [x] Arrhythmia
- [ ] Chest Pain
- [ ] MI
- [ ] CAD
- [x] HTN
- [ ] PVD
- [ ] Murmur
- [ ] Cancer: (Specify Type) In remission? ○ Yes ● No
- [x] Osteoathritis/DJD: (Specify site affected) Bilateral knee
- [ ] Joint Replacement: (Specify joint)
- [ ] Rheumatoid Arthritis
- [x] Gait Problems
- [ ] Fractures
- [x] Falls
- [ ] CVA
- [ ] TIA
- [ ] MS
- [ ] Hemiplegia
- [ ] Seizures
- [ ] Headaches
- [ ] Dizziness/Vertigo
- [ ] IBS
- [ ] Crohn's Disease
- [ ] Diverticulitis/Diverticulosis
- [ ] Constipation
- [ ] Diarrhea
- [ ] Fecal Incontinence
- [ ] Liver/Gallbladder Problems: (Specify)
- [ ] Depression
- [ ] Anxiety
- [ ] Dementia
- [ ] Substance Abuse: (Specify)
- [ ] Alzheimer's
- [ ] Mental Disorder: (Specify)
- [ ] Pressure Ulcer
- [ ] Stasis Ulcer
- [ ] Diabetic Ulcer
- [ ] Trauma Wound
- [ ] Other: (Specify)
- [x] Chronic Kidney Disease
- [ ] Renal Failure
- [ ] Dialysis
- [ ] Anemia
- [ ] Abnormal Coagulation
- [ ] Blood Clots
- [ ] Diabetes
- [x] Thyroid Problems
- [x] COPD
- [ ] Asthma
- [ ] Emphysema
- [ ] Chronic Obstructive Bronchitis
- [ ] Chronic Obstructive Asthma
- [x] Urinary Incontinence
- [ ] Urinary Retention
- [x] BPH
- [ ] Recent/Frequent UTI
- [ ] Tuberculosis
- [ ] Hepatitis: (Specify)
- [ ] Infectious Disease: (Specify)
- [ ] Tobacco Dependence

Type	Amount	Length of Time Used

Past Surgical History
tonsillectomy in childhood; cardiac catheterization in 2015 at Southern Hills Hospital

Most Recent Immunization

				Date
Pneumonia	○ Yes	● No	○ Unknown	mm/dd/yyyy
Flu	○ Yes	● No	○ Unknown	mm/dd/yyyy
Tetanus	○ Yes	● No	○ Unknown	mm/dd/yyyy
TB	○ Yes	○ No	● Unknown	mm/dd/yyyy
TB Exposure	○ Yes	○ No	● Unknown	mm/dd/yyyy

Hepatitis B
◯ Yes ◯ No ● Unknown Date mm/dd/yyyy

Additional Immunizations

Immunization Name
◯ Yes ◯ No ● Unknown Date mm/dd/yyyy

Immunization Name
◯ Yes ◯ No ● Unknown Date mm/dd/yyyy

Comments

Vaccination Record

1st Dose COVID-19
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

2nd Dose COVID-19
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

Booster 1
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

Booster 2
Product Name/Manufacturer (Lot No.) **Date** mm/dd/yyyy **Healthcare Professional or Clinic Site**

Health Screening

Last Cholesterol Level mm/dd/yyyy
Last Mammogram mm/dd/yyyy

Does patient perform monthly self breast exam? ◯ Yes ◯ No
Last Pap Smear mm/dd/yyyy
Last PSA mm/dd/yyyy

Last Prostate Exam mm/dd/yyyy
Last Colonoscopy mm/dd/yyyy

Fire Assessment for Patient with Oxygen

☐ Patient is not using oxygen

Does patient have No Smoking signs posted?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Does patient or anyone in the home smoke with oxygen in use?	◯ Yes	● No
• **Are the Patient and Caregiver educated regarding this?**	◯ Yes	◯ No
Are smoke detectors present and working properly?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Does patient have a properly functioning fire extinguisher?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Are oxygen cylinders stored properly?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Are all electrical cords near oxygen intact and free for fraying?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Does patient have an evacuation plan in case of fire?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Are all cleaning fluids and aerosols stored away from oxygen, and not used when oxygen is in use?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Does patient refrain from using petroleum products around oxygen?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No
Does patient only used water-based body and lip moisturizers?	● Yes	◯ No
• **Are the Patient and Caregiver educated regarding this?**	● Yes	◯ No

Comments

CARE PLAN (PROBLEMS / GOALS / INTERVENTIONS)

Patient Care Plan / Goals / Interventions:

Problem: Vital Signs
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient's vital signs will be within normal limits.
Interventions: Assigned Staff: SN
Check O2 saturation every visit for any shortness of breath on exertion, fatigue/weakness. Check VS every visit and report to PMD if patient vitals are not within the parameters.
Progress Toward Goals:
No Progress Yet

Problem: Cardiopulmonary
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Demonstrate pursed-lip breathing exercises, deep breathing exercises, and energy-conserving measures.

Interventions: Assigned Staff: SN
Review patient/caregiver on methods to conserve energy, frequent rest periods, sitting instead of standing to perform tasks, and performing slower, purposeful movements which will limit fatigue. Instruct patient/caregiver on pursed lip breathing exercises, deep breathing exercises to alleviate dyspnea.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from signs/symptoms of cardiac and respiratory distress during the certification period. Verbalize understanding of symptoms of cardiac complications and when to call 911.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver on sodium limitations, managing stress, avoiding smoking/alcohol intake, and compliance with prescribed medications to control blood pressure. Encourage the patient/caregiver to provide a quiet environment, limiting stimuli to promote rest. Advise the patient to get up slowly to offset the feeling of dizziness. Instruct patient/caregiver to notify SN/MD for signs/symptoms of cardiac and respiratory distress like chest pain, cyanosis, dependent edema, jugular vein distention, palpitations, wheezing, increased respiratory rate, anxiety, headache, lightheadedness, and shortness of breath.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from cardiopulmonary distress during the care episode. Patient's pulse and blood pressure will be within established parameters.
Interventions: Assigned Staff: SN
Reinforce patient/caregiver to check and record blood pressure, including heart rate. Assess cardiopulmonary status every visit.
Progress Toward Goals:
No Progress Yet

Problem: Bilateral primary osteoarthritis of knee
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce pain and improve mobility in the knees
Interventions: Assigned Staff: SN
Utilize heat and cold therapy to manage pain and swelling in the knees.
Progress Toward Goals:
No Progress Yet
Regularly assess pain levels and functional mobility to adjust the care plan as needed.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance activities of daily living (ADLs) and independence
Interventions: Assigned Staff: SN
Conduct home safety assessments to minimize fall risks and enhance mobility.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-ups to monitor progress and make necessary adjustments.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Educate patient and family about dietary changes to support joint health
Interventions: Assigned Staff: SN
Provide dietary counseling focused on anti-inflammatory foods and weight management.
Progress Toward Goals:
No Progress Yet
Encourage hydration and its role in joint health and overall well-being.
Progress Toward Goals:
No Progress Yet
Monitor the patient’s weight and adjust the diet plan as necessary.
Progress Toward Goals:
No Progress Yet

Problem: Lymphedema, not elsewhere classified
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce swelling in affected limb
Interventions: Assigned Staff: SN
Educate the patient and family on the importance of elevation of the affected limb.
Progress Toward Goals:
No Progress Yet
Utilize compression garments to manage swelling.
Progress Toward Goals:
No Progress Yet
Monitor and assess limb measurements regularly to track progress.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance patient and family understanding of lymphedema management.
Interventions: Assigned Staff: SN
Provide educational materials on lymphedema and self-care techniques.
Progress Toward Goals:
No Progress Yet
Conduct family meetings to discuss care plans and expectations.
Progress Toward Goals:
No Progress Yet
Facilitate support groups or counseling for emotional support.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Prevent complications associated with lymphedema.
Interventions: Assigned Staff: SN
Educate on proper skin care and hygiene in the affected area.
Progress Toward Goals:
No Progress Yet
Coordinate care with healthcare providers for any comorbid conditions.
Progress Toward Goals:
No Progress Yet
Develop a personalized care plan that includes emergency contacts and procedures.
Progress Toward Goals:
No Progress Yet

Problem: Hypertensive chronic kidney disease with stages 1-4/unsp chronic kidney disease
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain blood pressure within normal ranges.
Interventions: Assigned Staff: SN
Monitor and record blood pressure daily.
Progress Toward Goals:
No Progress Yet
Provide education on blood pressure management and medication adherence.
Progress Toward Goals:
No Progress Yet
Develop a personalized medication schedule for antihypertensive drugs.

Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Prevent progression of chronic kidney disease.
Interventions: Assigned Staff: SN
Assess kidney function through regular lab testing.
Progress Toward Goals:
No Progress Yet
Educate the patient and family about the importance of hydration and fluid management.
Progress Toward Goals:
No Progress Yet
Encourage regular follow-up appointments with a nephrologist.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance overall physical health and mobility.
Interventions: Assigned Staff: SN
Monitor and reassess physical capabilities regularly.
Progress Toward Goals:
No Progress Yet

Problem: Chronic kidney disease, stage 3a
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Maintain stable kidney function and prevent progression of the disease.
Interventions: Assigned Staff: SN
Monitor blood pressure and renal function through regular assessments.
Progress Toward Goals:
No Progress Yet
Educate the patient and family about the importance of adhering to prescribed medications and diet.
Progress Toward Goals:
No Progress Yet
Facilitate regular follow-up appointments with the nephrologist.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance patient and family education regarding chronic kidney disease management.
Interventions: Assigned Staff: SN
Conduct educational sessions for the patient and family about CKD and treatment options.
Progress Toward Goals:
No Progress Yet
Provide resources and materials on CKD management to the patient and family.
Progress Toward Goals:
No Progress Yet
Assess the patient's understanding of their condition and address any misconceptions.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage co-morbid conditions effectively to reduce the burden on kidney function.
Interventions: Assigned Staff: SN
Coordinate care for diabetes or hypertension management if applicable.
Progress Toward Goals:
No Progress Yet
Implement medication management strategies to avoid nephrotoxic drugs.
Progress Toward Goals:
No Progress Yet
Schedule regular lab tests to monitor electrolyte levels and kidney function.
Progress Toward Goals:
No Progress Yet

Problem: Chronic obstructive pulmonary disease, unspecified
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve respiratory function and reduce symptoms
Interventions: Assigned Staff: SN
Conduct regular pulmonary function tests to monitor lung capacity and function.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on proper inhaler techniques and medication adherence.
Progress Toward Goals:
No Progress Yet
Monitor environmental triggers that may exacerbate respiratory issues and provide avoidance strategies.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance overall physical endurance and strength
Interventions: Assigned Staff: SN
Conduct regular mobility assessments to adjust exercises as needed.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage nutritional needs to support respiratory health
Interventions: Assigned Staff: SN
Assess nutritional intake and provide dietary recommendations to support lung health.
Progress Toward Goals:
No Progress Yet
Monitor weight and nutritional status regularly to detect changes early.
Progress Toward Goals:
No Progress Yet

Problem: Chronic respiratory failure with hypoxia.
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve oxygenation and respiratory function.
Interventions: Assigned Staff: SN
Administer supplemental oxygen as prescribed to maintain SpO2 above 92%.
Progress Toward Goals:
No Progress Yet
Monitor respiratory status and oxygen saturation levels regularly.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on proper use of oxygen equipment.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance the patient's overall physical endurance.
Interventions: Assigned Staff: SN
Monitor and adjust the exercise program based on patient's tolerance.
Progress Toward Goals:
No Progress Yet
Provide education to the family on recognizing signs of fatigue.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Manage and minimize exacerbations of respiratory failure.
Interventions: Assigned Staff: SN
Educate the patient and family on medication management and adherence.
Progress Toward Goals:
No Progress Yet
Schedule regular follow-up visits to monitor the patient's condition.
Progress Toward Goals:
No Progress Yet

Problem: Fall Risk
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Reduce the risk of falls in the home environment.
Interventions: Assigned Staff: SN
Conduct a home safety assessment to identify and mitigate fall hazards.
Progress Toward Goals:
No Progress Yet
Educate the patient and family on fall prevention strategies.
Progress Toward Goals:
No Progress Yet
Review and adjust medications that may contribute to fall risk.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Enhance the patient's awareness of their surroundings and mobility.
Interventions: Assigned Staff: SN
Provide education on recognizing and reporting symptoms of dizziness or instability.
Progress Toward Goals:
No Progress Yet
Encourage regular eye exams to assess vision changes that may affect balance.
Progress Toward Goals:
No Progress Yet
Monitor and document any fall incidents to adjust the care plan accordingly.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Improve the patient's strength and balance through targeted exercises.
Interventions: Assigned Staff: SN
Schedule regular follow-up visits to assess progress and modify the exercise plan.
Progress Toward Goals:
No Progress Yet

Problem: Pressure Ulcer Prevention
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Verbalize understanding of the importance of pressure-relieving and proper incontinence care measures.
Interventions: Assigned Staff: SN
Instruct caregiver to change patient's pads/underwear frequently or as needed when heavily soiled to prevent skin breakdown related to excessive moisture from urine/stool. Instruct patient/caregiver to cleanse skin with mild soap and water then pat dry thoroughly after every incontinence. Instruct patient to wipe perineum from front to back to prevent infection. Instruct patient/caregiver on pressure-relieving measures such as repositioning every 2 hours, float heels, pad bony prominences and methods to reduce friction and shear.
Progress Toward Goals:
No Progress Yet

Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
The patient's skin remains intact.
Interventions: Assigned Staff: SN
Instruct patient/caregiver to keep skin clean and dry to prevent skin breakdown. Instruct patient/caregiver to inspect skin daily and notify SN/MD for redness or open sores. Assess skin for breakdown due to risk of pressure ulcer development.
Progress Toward Goals:
No Progress Yet

Problem: Rehospitalization Risk and Medication Management
Goals: Target Completion: 4 Week(s) Term: Long-term Status: Ongoing
Patient will be free from side effects and adverse medication reactions during the entire episode. Patient/Caregiver will demonstrate medication safety and compliance. Patients will have no re-hospitalizations.
Interventions: Assigned Staff: SN
Identify Medication Interactions and evaluate patient/family/caregiver(s) understanding of medication compliance.
Progress Toward Goals:
No Progress Yet

Ongoing Care Plans within OASIS date (03/30/2026):

Assigned Staff: Berberova, Emiliya RN

Electronically Signed by Berberova, Emiliya RN 03/30/2026